



higher education
& training

Department:
Higher Education and Training
REPUBLIC OF SOUTH AFRICA



UNIVERSITIES
SOUTH AFRICA



**Published by the Higher Education and
Training HIV/AIDS Programme**
April 2016

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About the Higher Education and Training HIV/AIDS Programme

Overview

The Higher Education and Training HIV/AIDS (HEAIDS) Programme is a national facility that seeks to reduce the HIV epidemic and its impact on society by developing and supporting programmes at public universities and technical and vocational education and training (TVET) colleges.

HEAIDS is an initiative of the Department of Higher Education and Training (DHET) and is undertaken by Universities South Africa (USAf, the representative body of public universities, previously known as Higher Education South Africa) and the South African College Principals Organisation (SACPO).

In 2015 it was primarily funded from the South African fiscus through an allocation that flowed through the DHET, grant funding from the National Skills Fund and membership contributions from public universities and colleges. HEAIDS also benefited from substantial funding from the Global Fund to Fight AIDS, TB and Malaria, and support from the German Development Agency, GIZ.

HEAIDS is accountable to the DHET, to all public universities (represented by USAf) and all public TVET colleges (represented by SACPO). These entities play a significant role in the Advisory Board, which guides and oversees the work of HEAIDS, and participate in the Executive Committee of the board. The board is chaired by Dr Max Price, Vice-Chancellor of the University of Cape Town. A full list of board members, who are drawn from a wide range of organisations, appears in Section 3.





Message from the Deputy Minister of Higher Education and Training

Mduduzi Manana

Deputy Minister of Higher Education and Training

The 2015 review of the Higher Education and Training HIV/AIDS Programme (HEAIDS) tells a story of sustained progress in expanding the programme across the sector and strengthening HIV and health interventions on many campuses.

I would like to express my appreciation to all concerned – in the HEAIDS Programme office, at the hundreds of campuses involved, and in our generous partner organisations that provide funding and operational support.

I would also like to assure you of the continued support and deep interest of the Ministry of Higher Education and Training in the programme.

Since the establishment of a separate portfolio for higher education, it has been possible for government to invest more time and resources in the HEAIDS initiative.

In 2015 I had the opportunity to take part in several HEAIDS First Things First activations. These usually take place at a technical and vocational education and training (TVET) college virtually every month. I had the privilege of witnessing first-hand the co-operation that brings activations to fruition. The realities that we encountered at college level and the enthusiasm of management, staff and students left a lasting impression. The experience served to deepen my understanding of the impact that social factors have on the achievement of our higher education goals.

The HIV epidemic has been a harsh but effective teacher. It compelled decision-makers in the higher education sector to sit up and take notice of the link between the health of students and staff and the performance of the higher education sector. In fact, it went beyond this: it increased our understanding of the impact of social and economic factors on the health of individuals and communities, and it left us in little doubt that a health catastrophe – if it occurred – would inevitably reverse many of the development gains made since 1994.

HEAIDS was a response to these lessons and we embrace HEAIDS as an education function because we have not forgotten the need to respond to our students' and staff members' needs in a more holistic way if we want education outcomes of the highest order.

Ultimately, the HEAIDS Programme is intended to pay dividends in terms of better student pass rates, lower drop-out rates, and higher throughput rates. It should also have a ripple effect in other economic sectors as universities and colleges produce graduates who understand the impact of health and wellness on the world of work and the productivity of the national workforce.

More immediately, the HEAIDS Programme is making a contribution to youth development through its emphasis on peer education as a platform for delivering HIV and health interventions. Post-school education should be much more than an extended academic experience. In order to prepare students for a role in the real world, institutions should expose them to opportunities for service, responsibility-taking, collaboration and for the frustrations and rewards of working with and for others. HEAIDS makes a substantial contribution to the ability of institutions to offer students opportunities for this kind of personal growth.

The Department of Higher Education and Training (DHET) therefore values the HEAIDS Programme and guarantees its support going forward. The programme can count on financial support from the National Budget to cover its core functions during the 2016/17 financial year. Equally importantly, it can depend on our engaged participation in its advisory structures and its stakeholder and public events, and our influence as an advocacy partner in the cause of improved student health.

A handwritten signature in black ink, consisting of a circular loop followed by a horizontal line extending to the right.

Mduduzi Manana

Deputy Minister of Higher Education and Training



Foreword by the Chairperson of the HEAIDS Advisory Board

Max Price
Chairperson
HEAIDS Advisory Board

An annual review inevitably sets in motion a process of reflection and taking stock. So I invite readers of the HEAIDS Annual Review for 2015 to pause with me and consider the programme's role in the wider context of the national HIV, TB and STI response and the changes that are in the making in the higher education environment.

Our starting point is the knowledge that HEAIDS has made considerable progress in expanding HIV, tuberculosis (TB) and sexually transmitted infections (STI) prevention interventions to public universities and colleges across the country and facilitating access to care and treatment for students and staff. It has succeeded in mobilising students as agents of health promotion. It has also devised new approaches to the elusive challenge of incorporating HIV content into the curricula taught at universities and colleges.

In short, HEAIDS and its participating institutions constitute a reliable and valued partner in the national effort to reverse the course of arguably the most destructive epidemic in living memory. The programme has shown that it has the capacity both to sustain and to innovate.

In order to build on this track record, HEAIDS participants at all levels need to be alert to the current challenges and opportunities. As it moves forward in 2016, HEAIDS will find that two critical crossroads lie just ahead.

The first crossroad involves a choice whether to capitalise on the great gains that have been made towards overcoming the HIV epidemic, or to live with the current status quo for a while and ultimately lose ground against the relentless virus.

While South Africa saved some 300 000 people from early death through its antiretroviral treatment programme in 2014 (according to UNAIDS estimates) and added more than eight years to life expectancy between 2005 and 2013, tens of thousands continue to die from AIDS-related factors each year. And although new HIV infections have dropped substantially, about 300 000 people still become infected each year in this country (UNAIDS).

The message is clear: we cannot mistake progress for victory and our efforts will have to be greatly intensified to secure victory. It is this commitment that HEAIDS will have to make in the near future.

What this will entail is too complex for us to unpack here, but it will certainly include:

- Focusing increasingly on the quality and impact of HEAIDS prevention interventions, even as we expand their scope
- Striving for the optimal balance between behaviour change and biomedical forms of HIV prevention within the general approach of combination prevention
- Contributing to the search for HIV prevention approaches that really work for the student age group, as evidence from multiple clinical trials shows the difficulty of achieving sustained use of prevention methods at this age.

The second crossroad involves choices that might carry important advantages for the HEAIDS Programme. These centre on the question of how to overcome the social and economic barriers that undermine equity in higher education and inhibit the role of universities as instruments of upward mobility.

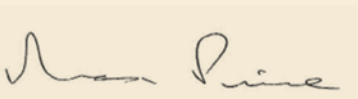
The student protests that exploded in the last quarter of 2015 and continued into 2016 revealed a degree of deprivation among students that was not common knowledge. The reality of hunger, homelessness and vulnerability to exploitation within the student population has been brought into the open and there are various moves to address these.

HEAIDS has a vital interest in the resolution of these poverty-related problems. HEAIDS national surveys of universities and colleges indicated that socio-economic

factors increased the HIV risk-profile of sections of the student population. While the National Strategic Plan on HIV, TB and STIs directs us to address this type of risk through structural interventions, such remedies demand powerful influence and very considerable financial input.

At this moment there is a unique convergence of interests focused on overcoming the socio-economic barriers to young South Africans entering universities, studying in secure circumstances, and enjoying an equal chance of graduating. Depending on how the solutions play out, some structural risk factors for HIV – and other health conditions – could be addressed in the course of this.

The purpose of this excursion to the crossroads was to highlight the complexity of the task that faces HEAIDS in 2016 and the years beyond. The intention was also to prepare the ground for a call to action to the multitude of organisations and individuals who contribute to the HEAIDS Programme to go the extra mile to make a fine programme into a great one. The situation demands that we up our game and we cannot fail to make the right response.



Max Price
Chairperson
HEAIDS Advisory Board



Message from the Deputy Chairperson of the HEAIDS Advisory Board

Hellen Ntlatleng
Deputy Chairperson
HEAIDS Advisory Board

My association with the Higher Education and Training HIV/AIDS (HEAIDS) Programme came about because I was President of the South African College Principals Organisation (SACPO) when the technical and vocational education and training (TVET) sub-sector joined the HEAIDS family in 2014.

For those of us whose hearts lie with TVET colleges, this inclusion was long overdue. Most of our students come from humble circumstances and many live in poorly developed parts of our country. They are hungry for a better life – but at the same time they are extremely vulnerable to the many traps that poverty lays for them, including HIV. This high risk profile was confirmed in 2015 when HEAIDS, through the Human Sciences Research Council, completed a national survey of knowledge, attitudes and behaviour of TVET students and staff in relation to HIV, TB and related matters.

SACPO has therefore welcomed wholeheartedly the opportunity to join HEAIDS. In 2014, colleges began to feel the benefits of HEAIDS membership but in 2015 we knew without a doubt that HEAIDS was fully engaged in the sub-sector. The Annual Review for 2015 sets out very clearly the increase in the scale of HIV and related activities that occurred at TVET colleges in the second year of implementation.

The success that has been recorded is largely due to an ingredient that is often under-valued and therefore neglected. That ingredient is partnership. Too many organisations to mention have played a part in bringing multi-faceted First Things First activations to campuses across the country, in increasing HIV testing among students, in making millions of condoms accessible, and developing health awareness among students.

Without these partners the large number of activities reported in this document would never have happened. While others may see dependence on partners as a second-best solution, I see it as a triumph of the collaborative spirit and a victory for prudent use of resources.

That said, building partnerships is often a messy business and it requires patience. In the real world, partners do not always play together like a well-rehearsed orchestra. So I would like to salute all those who persevered and applied themselves to the building of partnerships that have enabled our colleges to become active players in the fight against HIV and TB. We recognise your achievements during 2015 and have the highest expectations for 2016!

SACPO will continue to provide wholehearted support to the HEAIDS Programme in the interests of our students' health and their educational success.

A handwritten signature in black ink, appearing to be 'Hellen Ntlatleng'.

Hellen Ntlatleng
Deputy Chairperson
HEAIDS Advisory Board



SECTION 1: OVERVIEW BY THE DIRECTOR

Dr Ramneek Ahluwalia
Director: HEAIDS

1.1 Introduction

During the course of 2015 the Higher Education and Training HIV/AIDS (HEAIDS) Programme achieved impressive gains in programme output at institution level. This growth was due both to technical and vocational education and training (TVET) colleges playing a stronger role, following their incorporation into HEAIDS just a year earlier, and to universities gaining substantial ground as more resources became available for the programme. Some key achievements were:

- The number of students tested for HIV in 2015 was 79% higher than in the previous year. The combined effort of universities and colleges not only exceeded the 100 000-test mark for the first time, but did so convincingly with a total of 174 000 tests performed
- Most HIV testing of students occurred during HEAIDS First Things First (FTF) activations and the number of activations topped the 2014 figure by 58%, thanks to an enormous effort by universities, colleges, a range of partner organisations, and the HEAIDS national and provincial teams
- Screening for tuberculosis (TB) and sexually transmitted infections (STIs) during activations was almost as extensive as HIV testing, and actually showed a somewhat higher year-on-year increase
- Nearly 25 million male condoms were distributed in the higher education sector, mainly as a result of the Department of Health's decision to pilot the introduction of its new coloured, scented condoms in this sector. The 2014 total of 114 650 male condoms seems almost insignificant in comparison.

This enthusiastic response from students speaks to the success of the FTF model in promoting a health-seeking culture on campus which enables young people to assume responsibility for their own wellbeing. The UNAIDS 90:90:90 campaign challenges us to ensure 90% of the population know their HIV status and, if needed, access effective antiretroviral treatment. Though the higher education sector still has a long way to go, its HIV testing numbers have begun to look significant in terms of the total population attending higher education institutions. Nationwide the HEAIDS Programme enabled about one in ten members of this population to take an HIV test, and in some provinces, for example the Northern Cape and Limpopo, the ratio was closer to one in five.

The explosion in programme performance also owes much to major advances in terms of the support HEAIDS was able to offer institutions and the resources it made available to them. These included:

- An increase of 126% (from 2 162 in 2014 to 4 895 in 2015) in peer educators trained to mobilise students and conduct health promotion on campus
- Stronger support to colleges through HEAIDS provincial co-ordinators who increased their visits to colleges by 40% and conducted no fewer than 580 visits in the course of the year
- The mobilisation of considerable funding from the National Skills Fund (NSF) and the Global Fund to Fight HIV/AIDS, TB and Malaria to enable individual universities to strengthen HIV programme delivery and to accelerate HIV curriculum development. All universities benefited from this effort and most

received a total of R1 million or more through direct grants, and an amount channelled through the Networking HIV and AIDS Community of South Africa (NACOSA)

- Measurable progress in terms of building capacity among academic staff to develop and deliver HIV content through curricula offered by different faculties and disciplines. A total of 402 academics participated in curriculum development workshops during 2015, thanks to the NSF grant made for this area of work
- Expansion in the use of campus and community radio stations, under the Future Beats Programme, to take peer education to a wider audience. By increasing the number of participating radio stations to 11, Future Beats more than doubled the size of its audience to an estimated 770 000 listeners and it trained 122 volunteer journalists during the course of 2015.

HEAIDS' total income for 2015 amounted to R35.2 million, nearly two-thirds of which derived from funds allocated by the National Treasury to the higher education sector. In keeping with sound principles of sustainability, HEAIDS has reduced its dependence on donor funding and diversified its sources of funding to a certain degree.

However, the formal budget of HEAIDS simply does not reflect that magnitude of resources available to this programme. It is impossible to quantify the donations in kind that universities and colleges received in 2015 to enhance their HIV, TB and STI programmes. Certainly, these contributions from partner organisations, mostly in the public health sector, but also from NGOs, amounted to millions of rands' worth of medical consumables and health professionals' service. These partners and our major donors have become indispensable to the daily activities that take place under the umbrella of HEAIDS.

All of this is driven and made possible by the high-level buy-in and visible backing HEAIDS enjoys from the Ministry and Department of Higher Education and Training, from leadership in provincial government and provincial AIDS councils, from Universities South Africa (USAf) and its member universities, and from the South African College Principals Organisation (SACPO) and the colleges they represent.

This introduction to the performance of HEAIDS in 2015 would be incomplete without reference to the long-term perspective which locates the programme firmly within the vision for higher education and its contribution to the economic development of our country. Ultimately, the contribution of HEAIDS will be measured in terms of its impact on the efficiency and effectiveness of the higher education system, measured in student throughput rates and the competency of graduates to meet the development challenges of our society.

1.2 Contributing to a concerted national effort

HEAIDS participates in the South African National AIDS Council (SANAC) and its activities are guided by the National Strategic Plan on HIV, STIs and TB 2012–2016. The plan's goals for the period 2012–2016 are to:

- Halve the number of new HIV infections
- Ensure that at least 80% of eligible people receive antiretroviral treatment (ART)
- At least 70% of those receiving ART are alive and still on treatment after five years
- Halve the number of TB infections and deaths due to TB
- Ensure the rights of people living with HIV are protected
- Halve the stigma related to HIV and TB.

The National Development Plan, the 2013 White Paper on Post-School Education and Training, and the global Sustainable Development Goals have also provided significant direction to HEAIDS' policies, strategies and programmes.

Because HEAIDS serves an overwhelmingly young population, it has always placed considerable emphasis on prevention of HIV. The prevalence of HIV in the South African population – among women in particular – climbs most steeply in the late teens and early 20s. HEAIDS' desire to steer our students safely through this critical age has sharpened the programme's focus on prevention.

The programme follows the National Strategic Plan on HIV, TB and STIs in adopting the approach of combination prevention, which seeks to match different methods of HIV prevention to the particular



circumstances and needs of specific social groups or populations.

In pursuit of effective prevention strategies and in order to reduce the stigma that is still associated with HIV and TB, HEAIDS has taken the view that it is helpful to address HIV and TB within a broader health promotion approach. Accordingly, many of the programme's HIV and TB interventions take place alongside promotion of healthy lifestyles and screening for various non-communicable health conditions.

1.3 One strategy, unique styles of implementation

In enabling the higher education sector to develop its response to the daunting challenges of the HIV epidemic, HEAIDS has produced broad policy frameworks, programmes and a model of practice (see section 1.4) that can be adapted to the unique circumstances of each institution. It is not far-fetched to describe HEAIDS as 400 campus-level programmes unfolding under the umbrella of a national programme.

Even when HEAIDS only comprised public universities, each institution had its particular character and culture, and there were variations even among campuses within the same university. The diversity within the programme increased further with the inclusion of TVET colleges and the formation of three new universities – Sol Plaatje University, Sefako Makgatho University, and the University of Mpumalanga – which face different challenges from the large, well-established universities.

TVET colleges, as a group, differ in some respects from public universities. The college campuses are generally smaller. They are more dispersed across the country and include many more rural locations. In contrast to

universities, colleges have few residential students and the average age of students is slightly younger.

A study on HIV-related knowledge, attitudes and behaviour of TVET students and staff (see section on Research Projects) revealed that a high percentage of students came from disadvantaged circumstances: 29% said they often lack money for food and clothing, while 44% indicated they could only afford food and clothing, but little else.

During the course of 2015, two developments in particular brought the reality of diversity in the higher education sector strongly to the fore:

- The first was the publication of the *HEAIDS Survey on the Knowledge, Attitudes, Beliefs and Practices of Lesbian, Gay, Bisexual, Transgender and Intersex (LGBTI) Individuals on University Campuses*. This study enabled HEAIDS to appreciate the complexity of gender identities and sexual practices among students. It found that, although LGBTI students have much in common with other students, they have some heightened risk factors for HIV, and special needs in relation to HIV risk reduction and healthcare which are often not catered for.
- The second was the wave of student protests, characterised as the #FeesMustFall movement, which drew attention to the dire economic need experienced by many students. HEAIDS is alert to the fact that the struggle of these students to secure accommodation, buy food and pay tuition fees could translate to increased vulnerability to HIV infection.

These developments underscored the need for HEAIDS to assist institutions in responding sensitively to a range of student needs.

Table 1: Number of institutions, campuses, students and staff in university and TVET sub-sectors

	University sub-sector	TVET college sub-sector
Number of institutions	26	50
Number of campuses	110	244
Number of students	Approximately 1.3 million	Approximately 710 000

1.4 Overview of the sector served

The combined student enrolment of public universities and public TVET colleges was in the region of 2 million in 2015. Universities are fewer in number and larger, accounting collectively for about 1.3 million students. There were 50 TVET colleges with 244 campuses and a combined student population of about 710 000.

At this stage there is an asymmetry between universities and TVET colleges in the resources and expertise available to tackle the challenges of HIV, TB and STIs. The fact that HEAIDS is well into the second decade of its work with universities shows in the relative maturity of the response on most campuses.

In closing this gap, the smaller size of TVET colleges is both an advantage and a challenge. On the one hand, health education campaigns at colleges have the potential to be more close-up and personal – and therefore more impactful. On the other hand, few colleges can support dedicated, comprehensive health services on campus for students and staff and need to make alternative arrangements to meet the need.

1.5 The HEAIDS model of practice

Drawing on research and its years of experience in the sector, HEAIDS has forged a model of practice designed to meet HIV prevention, care and treatment needs, as well as some broader primary health needs of various sections of the student population.

At the centre of the model is the First Things First (FTF) Programme, which focuses on prevention and early detection of a range of health conditions and health risks. The underlying premise of this programme is that it is the first priority of every South African to look after his or her health.

Through FTF, HEAIDS aims to build a culture where youth take an interest in their health, become knowledgeable about it and incorporate testing and screening into the normal course of their lives. In this manner HEAIDS makes a specific contribution to the UNAIDS target of ensuring that 90% of people living with HIV know their status, 90% of these receive antiretroviral treatment, and 90% of those on treatment achieve viral suppression.

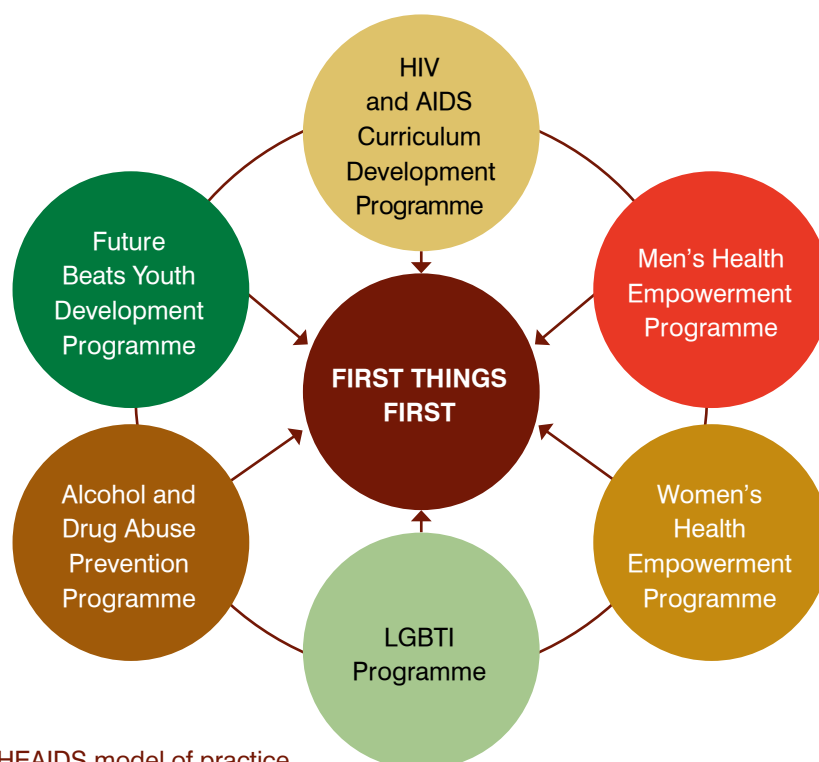


Figure 1: The HEAIDS model of practice

HEAIDS enables students to assume responsibility for their health by conducting FTF activations at all campuses. The goal is to undertake at least two activations a year on every campus and this target is within reach. Activations differ slightly from campus to campus but each one encompasses:

- Health promotion, including condom promotion
- Testing and screening for various health risks and health conditions
- Individual counselling of students in relation to health matters
- Referral for treatment and care, further testing and medical male circumcision.

Initially FTF focused exclusively on HIV, TB and STIs and only later began to include contraceptive services, non-communicable diseases (such as diabetes and hypertension) and certain forms of cancer.

When an FTF activation is requested by a university or college, this institution becomes the primary implementation partner. However, many other partners in government and civil society are necessary in order to offer a full package of interventions during the activation.

In addition to FTF, HEAIDS facilitates four focused programmes that are run by universities throughout the year and are just getting started at colleges. These programmes are advanced mainly through activities organised by a corps of peer educators that has been established on campuses. The content of each programme is suggested by its title:

- Women's Health Empowerment
- Men's Health Empowerment
- Drug and Alcohol Abuse Prevention Programme
- LGBTI Programme.

While peer educators are primarily responsible for convening discussion groups and dialogues related to these programmes, their efforts are reinforced

through two additional delivery platforms: the Future Beats Youth Development Programme, which uses radio and social media to engage students on relevant topics, and the Curriculum Development Programme, which strives to build student competencies on HIV (for later application in the workplace) through the curricula taught at universities and colleges.

In reality, FTF and the four additional programmes overlap and reinforce each other. Activations may include a specific emphasis on one of the other campaigns if this is especially relevant to the campus where the activation takes place.

1.6 The HEAIDS model at work

1.6.1 Campus activations

Both the institutions' demand for FTF activations and HEAIDS' capacity to deliver them increased significantly in 2015. The number of activations undertaken at university campuses was 44% higher than in 2014, while TVET campuses saw a 66% increase in activations.

Strong partnerships with provincial and district health authorities and non-governmental organisations are at the heart of the successful expansion of activations. HEAIDS would like to express its appreciation to all those individuals and organisations that have played a role.

Only 5% of planned activations in the University Programme failed to materialise and these were all due to student protests on campus. The TVET College Programme delivered 83% of activations requested. While disruption due to student protests was a major factor in non-delivery of planned activations, other organisational factors also played a role.

The 2015 national launch of FTF took place in Soweto and extensive media coverage projected it to hundreds of thousands of readers, TV viewers and radio listeners countrywide.

AN HIV PREVENTION FESTIVAL TO REMEMBER

Not many students can claim that the Minister of Health supplied their contraception. But three young women at the 2015 national launch of the First Things First (FTF) Programme had this distinction, as Minister Aaron Motsoaledi, who is a medical doctor, performed their contraceptive implants.

The promotion of this form of contraception, which had only recently become available in the public health service, was just one aspect of HIV prevention and sexual health featured at the launch. The event was a real festival of health promotion and screening – all shown live on national TV through an on-site broadcast of the show, *Morning Live*.

The event took place at the South West Gauteng College in Soweto. Each year the national launch of FTF sets the tone for hundreds of smaller activations at all campuses across the country. It showcases the partnerships which underpin activations and the key role played by peer educators in mobilising their fellow students and spreading awareness.

A special feature of the 2015 launch was the unveiling of the first government-supplied grape-coloured and -scented condoms prior to their mass distribution to universities and colleges across South Africa. The privilege of doing this was shared by Dr Motsoaledi and Deputy Minister of Higher Education and Training Mduzuzi Manana.

During the course of the day, 1 500 students participated in the formal programme and utilised the services on offer. For example, 581 students took an HIV test, 560 were screened for TB and 262 had their blood pressure checked. In addition, 31 young women had contraceptive implants done and 18 young men underwent circumcision.

The organisation of this launch was achieved through an unprecedented degree of partnership involving the Department of Higher Education and Training, the national and Gauteng Departments of Health, the host-college, important NGOs and several other institutions.



A small number of activations were designated as provincial launch activations. Rural locations were deliberately chosen in many cases in order to focus attention on the challenges these colleges faced. The level of interest generated by these activations was boosted by the participation of Deputy Minister of Higher Education and Training, Mduduzi Manana, provincial MECs for Health, mayors from nearby municipal areas and student leadership. There is a ceremonial component, where all leadership figures sign a pledge committing themselves to the fight against HIV, TB and STIs. The presence of journalists and resultant local media coverage communicates the activation to nearby communities.

In 2015, there were five provincial launches at:

- The Lebowakgomo campus of Waterberg TVET College in Limpopo
- The Ulundi campus of Mthashana TVET College in KwaZulu-Natal
- The Mthata campus of King Sabatha Dalindyebo TVET College in the Eastern Cape
- The Lichtenburg campus of Taletso TVET College in North West
- The Khayelitsha campus of False Bay TVET College in the Western Cape.



HEAIDS held five successful provincial FTF launches

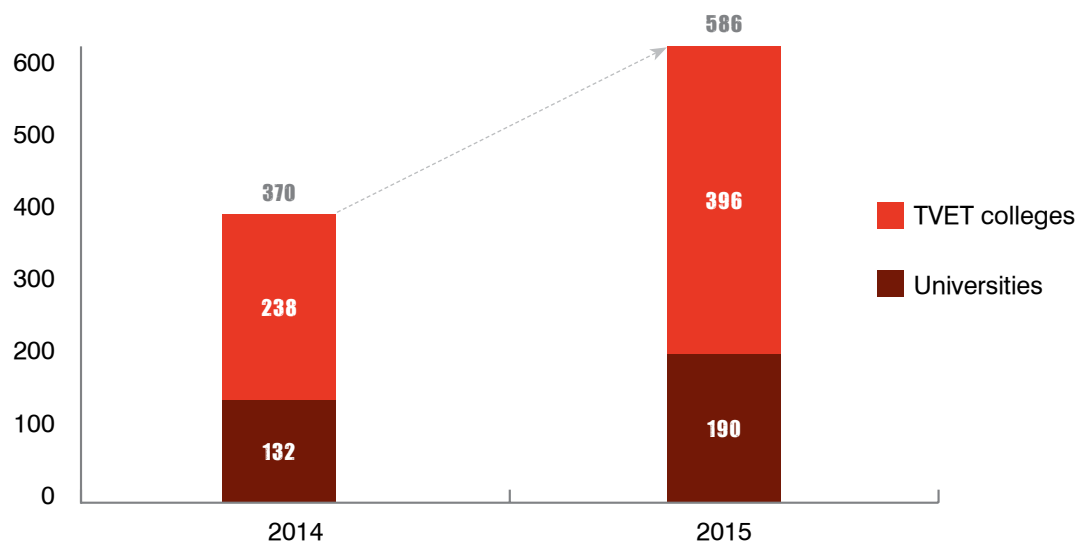


Figure 2: Number of activations at universities and TVET colleges in 2014 and 2015

1.6.2 HIV testing and screening for TB and STIs

The FTF approach has delivered substantial year-on-year increases in the number of students and staff testing for HIV and undergoing screening for TB and STIs (Figure 3). Analysis shows that 95% of HIV tests performed on campus occur in the context of activations and only 5% at health clinics at other times. Convenient access, visibility and an approach which presents testing as a lifestyle rather than a medical matter seem to contribute to the popularity of the FTF testing services.

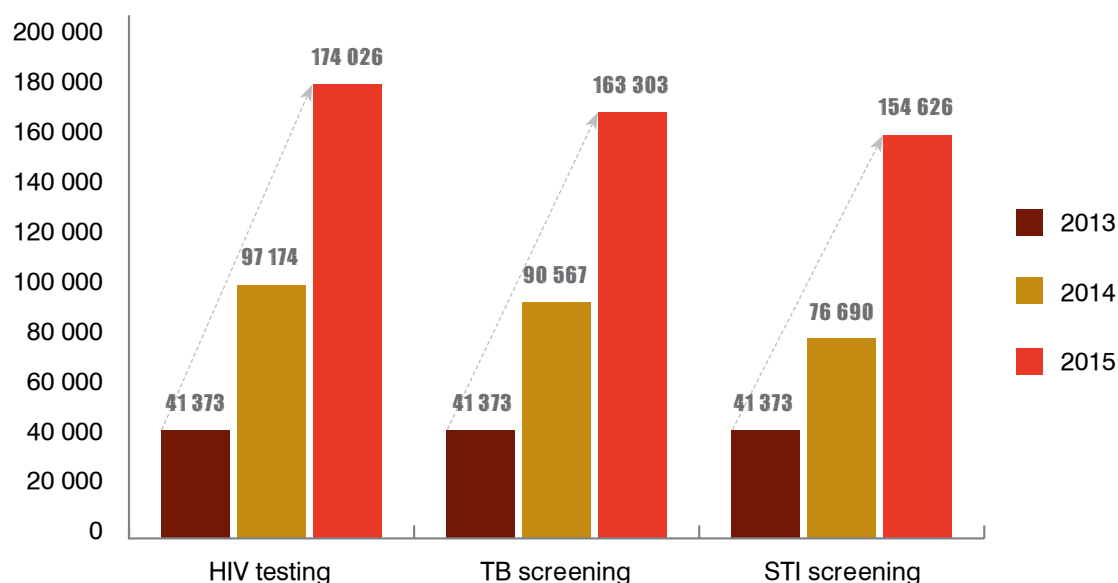


Figure 3: Numbers tested/screened at universities and colleges, 2013 to 2015

In 2015 the numbers of individuals tested for HIV and screened for TB and STIs were almost the same in the university sub-sector (88 607) and TVET college sub-sector (85 419). The achievement in the TVET sub-sector is particularly impressive: not only is this just the second year that HEAIDS has conducted activations in this sub-sector, but given the smaller enrolment at individual TVET campuses, substantially more activations were required to make up this total.

The extent of progress can also be measured in terms of the percentage of the total post-school

student population tested for HIV. Figure 4 indicates that in 2015 HEAIDS HIV counselling and testing (HCT) activities benefited 11% of the total number of students attending post-school institutions in South Africa. Coverage differed considerably across provinces, with two provinces (Limpopo and the Northern Cape) approaching a ratio of one in five students tested. However, the size of the post-school student population varies immensely by geographic area and some provinces have a much higher mountain to climb than others.

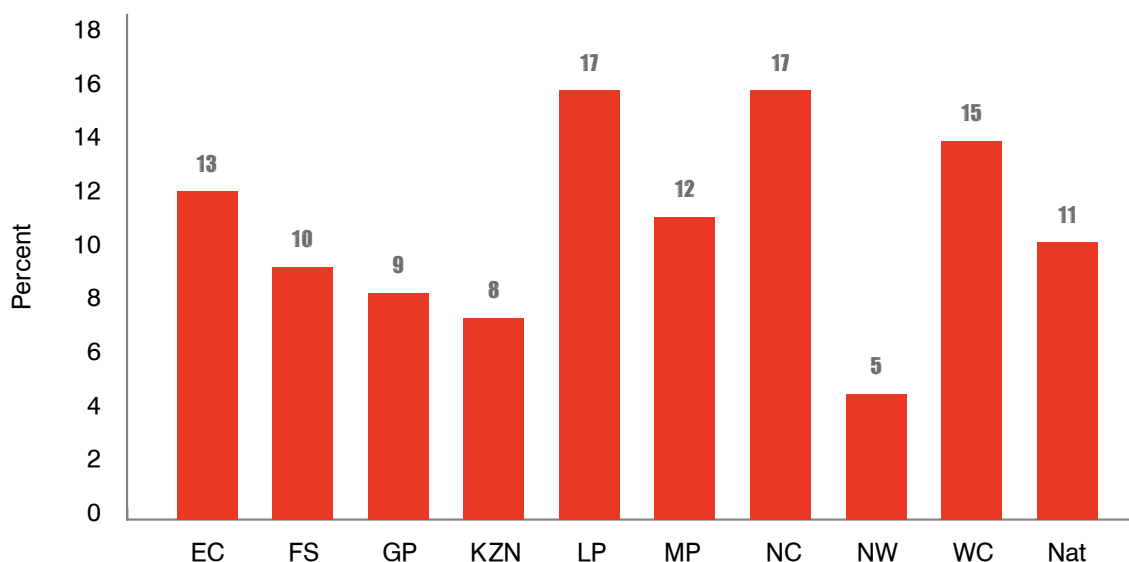


Figure 4: Percentage of all post-school students tested for HIV in 2015 by province

1.6.3 Condom distribution

The Department of Health partnered with the higher education and training sector for the introduction of the first coloured and scented government-sponsored condoms. This was designed as a pilot project to gauge the response of young people in advance of the marketing of a full range of coloured condoms in 2016.

The initiative was launched during the national FTF activation at the South West Gauteng College in Soweto in March.



Social marketing activities to promote the new government-issued grape-scented condoms in the higher education sector took place on 20 university and college campuses.

As a result of this initiative, 2015 saw a record of 20 million male condoms distributed, as well as some 42 000 female condoms and 450 000 tubes of lubricant. From May 2015 TVET colleges were included in the HEAIDS condom distribution system and it became possible to monitor stock moving into the colleges. The direct grant to universities from the Global Fund made it possible for institutions to manage the distribution of hugely increased condom supplies.

Qualitative research indicated that student perceptions of the coloured condoms were overwhelmingly favourable and most users felt that quality concerns about the old-style Choice-branded condoms had been resolved.

1.6.4 Peer education training

Peer education is widely regarded as one of the more successful forms of behaviour change communication. It is especially appropriate for reaching and influencing young people, who are growing up in a time of

unprecedented change in which the advice of older generations may seem less relevant than previously.

The primary reason for HEAIDS building a strong peer education foundation is the belief that educated young people, working in communities and settings where they have roots and a sense of belonging, constitute a powerful force for change and development.

HEAIDS more than doubled its investment in the training of peer educators in 2015, as Figure 5 indicates. The training covers key information and sensitisation to issues that may arise in the following areas:

- HIV, TB and STIs and the prevention and management of these infections
- The Women's Health Empowerment Programme
- The Men's Health Empowerment Programme
- The Programme for LGBTI Students
- The Programme to Prevent Alcohol and Drug Abuse.

During 2015, HEAIDS began to design a new training system in preparation for additional scale-up of peer education in 2016. Details of this system appear in the section on the TVET College Programme.

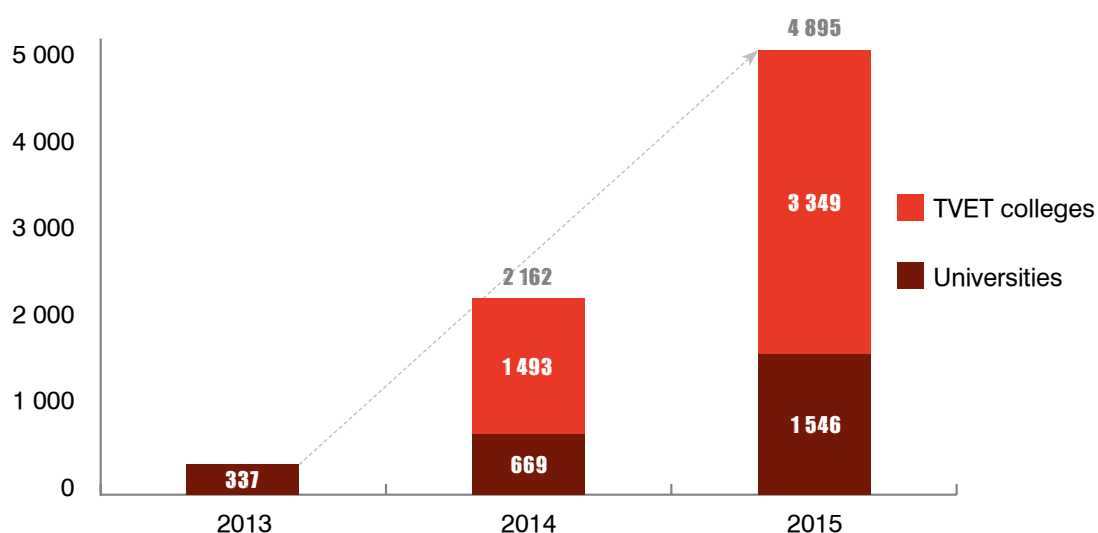


Figure 5: Number of peer educators trained at universities and colleges, 2013 to 2015

1.6.5 HIV Curriculum Development

From its inception, HEAIDS has been committed to utilising the formal teaching and learning programmes of higher education institutions to equip students with relevant HIV knowledge and skills that they could put to use in their personal lives and, after graduation, in their places of work. Employers in both the private and public sectors, as well as the Department of Labour, have identified a need for graduates who are able to respond to HIV and other health and wellness issues in the workplace. These factors are increasingly recognised as a constraint to productivity and economic growth.

The year 2015 marked something of a breakthrough in this area as the National Skills Fund (NSF) made a substantial three-year grant through HEAIDS to 20 universities and the TVET sub-sector as a whole to take HIV curriculum development forward.

For universities, this means that they have resources to provide academic staff with face-to-face capacity building opportunities. Academics have long requested such support in order to take on the teaching of HIV-related content in a relevant and competent manner. HEAIDS believes that investing in the development of HIV programme champions among teaching staff across institutions will create a more dynamic and sustainable programme.

By the end of 2015 no fewer than 402 members of the academic staff of universities had attended specialised workshops on developing and/or delivering content on HIV within the formal learning programme. These academics were drawn from a wide range of disciplines in several different faculties. HEAIDS believes in the power of peer-to-peer interaction, and it is apparent in this area of work as well: the conceptualisation and delivery of all workshops were undertaken by academics who have pioneered HIV curriculum development and are in a strong position to assist their colleagues.

The approach to TVET college curriculum development has been somewhat different. This is partly due to the recent entry of HEAIDS into the sub-sector, but also due to the kind of curricula taught at colleges. A baseline study was undertaken during 2015 to get to grips with the needs, opportunities and challenges of building HIV content into TVET teaching programmes.

This resulted in a decision to prioritise a limited number of work streams initially and focus on introducing HIV content into college teaching programmes within these work streams. The first three that will receive attention are Life Orientation, Mining and Agriculture. The Life Orientation Work Stream will be linked directly to the HEAIDS peer education platform, so practical learning and service to the college become mutually reinforcing activities. More detail on how the TVET work streams will get started are included in the HIV Curriculum Development section of this review.



HIGHLIGHTS FROM THE 2015 CONFERENCE ON AIDS AND STIS IN AFRICA (ICASA)

African researchers, programme managers and advocates in the field of HIV and STIs need a dedicated forum to discuss their work on the continent which bears the heaviest burden of these epidemics. ICASA, or the International Conference on AIDS and Sexually Transmitted Diseases in Africa, is that forum.

HEAIDS contributed wholeheartedly to the 18th session of ICASA, held in Harare in November 2015 under the theme, *AIDS in the Post-2015 Era: Linking Leadership, Science and Human Rights*.

Fielding a five-member team headed by Director Dr Ramneek Ahluwalia, HEAIDS went to ICASA with the aim of sharing its successes, learning from others working in relevant areas, supporting evidence-based approaches to HIV, and using the platform to mobilise additional support for HEAIDS.

HEAIDS organised and presented a satellite session on HIV prevention interventions in the post-school sector, in which it showcased some of the programme's formative research and the HEAIDS model of practice that centres on First Things First activations on campuses across South Africa.

In addition to Dr Ahluwalia the panel of presenters comprised Prof. Olive Shisana, then CEO of the Human Sciences Research Council; Dr Sheila Tlou, UNAIDS Representative for Southern and Eastern Africa; Ms Steve Letsike, Co-chair of the South African National AIDS Council; and Prof. Simba Rusakaniko of the Department of Community Medicine at the University of Zimbabwe.

HEAIDS Technical Advisor, Miriam Behrendt, gave a presentation on the Future Beats Youth Development Programme and its impact on HIV awareness levels. This formed part of a conference track devoted to technology and communication. Finally, HEAIDS Senior Programme Manager, Managa Pillay, delivered a poster presentation on the HEAIDS experience of integrating HIV content into the formal curricula of universities and colleges.

The vital link between research and programme development was evident across all HEAIDS presentations. This and HEAIDS' ability to talk on the basis of hard experience and real achievements drew an encouraging response.



1.6.6 Future Beats Youth Development Programme

The Future Beats Programme, which delivers “wholesale peer education” using radio as a platform, extended its presence on campuses and in communities in 2015 as it added five new radio stations to the original six that kicked off the intervention in 2014.

The essence of Future Beats is that it seeks to equip programme managers and volunteer journalists at radio stations to produce content that is broadly HIV-related and is accurate, insightful and exciting to student audiences. The programme encourages journalists to capitalise on the interactive nature of radio to facilitate dialogue on campus on important social issues related to HIV. It builds their ability to complement radio content with use of social media platforms in order to make the student listening experience more personal.

The number of campus and community radio journalists trained in 2015 rose to 122. By adding five new stations, the combined audience reached by the programme grew from an estimated 270 000 listeners to an estimated 770 000 listeners. For more details see the Future Beats Programme section.

1.7 Research as the foundation for action

HEAIDS has a strong track record of generating evidence as a tool for programme planning and in 2015 added to this body of research. Major developments were:

- **The release of the findings of the *HIV Knowledge, Attitudes and Behaviour Survey of Students and Staff at TVET Colleges***

The Minister of Higher Education and Training, Dr Blade Nzimande, headed up the bill at a satellite session of the South African AIDS Conference in June 2015 where the findings of this survey were presented and examined. The survey was commissioned by HEAIDS and conducted by the Human Sciences Research Council (HSRC). It provided valuable insights into the risk factors for HIV infection among students and staff, as well as establishing a strong need for information and services related to HIV, TB and sexual health. It also flagged challenging attitudes and perceptions

among students and staff which HEAIDS and the colleges would need to address when promoting HIV risk-reduction, sexual health, contraception and moderation in alcohol consumption.

- **The publication of the *Survey on Knowledge, Attitudes and Behaviours of Lesbian, Gay, Bisexual, Transsexual and Intersex (LGBTI) Individuals within Campus Communities Nationwide***

This ground-breaking report was launched at a special event at the University of South Africa in Pretoria in May 2015. This event was positioned as an advocacy opportunity, designed not only to share research findings, but also to mobilise support for implementation of key recommendations on how to create a more supportive campus environment for LGBTI students. The research project was commissioned and managed by HEAIDS in partnership with the Networking HIV and AIDS Community of SA (NACOSA), and was undertaken by a team at the University of Stellenbosch. Main findings of this study are summarised in the Research Programme section.

- **The completion of a *Situational Analysis/Baseline Study on the Nature and Extent of HIV Education at TVET Colleges***

The intention was to identify good practice and enabling mechanisms, as well as gaps and challenges to the teaching of HIV content. Funded by the NSF grant and undertaken jointly by the Centre for Education Policy Development and the SA Institute for Distance Learning, this study was seen as a tool to chart the way forward in strengthening HIV teaching in the sub-sector. The study was completed in 2015 and circulated to relevant stakeholders. HEAIDS plans to release the study to the public in 2016.

The ***Evaluation of the Pilot Phase of the Future Beats Youth Development Programme*** was conducted to establish whether campus radio and social media can contribute to the development of awareness of HIV and related social issues among students. The study was funded by HEAIDS and GIZ and undertaken by Wits School of Public Health. It was conducted between February 2014 and February 2015 and published as an ebook in July.

VISIBLE AND VOCAL AT SA AIDS 2015

HEAIDS offered delegates to the biennial South African AIDS Conference in Durban something interesting to see and quite a few things to talk about. The programme's participation in the conference included:

- An exhibition which showcased HEAIDS' programme work and provided opportunities to renew contact with existing stakeholders and meet newcomers
- A satellite session to present and reflect on the *Study of Knowledge, Attitudes and Behaviour (KAB) of Students and Staff Members in the TVET College Sub-sector*
- An event where Deputy Minister of Higher Education and Training, Mduzuzi Manana, formally launched the TVET sub-sector study
- A presentation on the Future Beats Youth Development Programme, delivered by Technical Advisor Miriam Behrendt under the title "Future Beats is bringing the beat back! Can campus radio and social media activate students to rediscover their risk?"

The Minister of Higher Education and Training, Dr Blade Nzimande, headed up an extensive panel of presenters in the satellite session. Other presenters were Dr Ahluwalia on behalf of HEAIDS, SACPO Chair Ms Hellen Ntlatleng, Department of Health Deputy Director-General Dr Yogan Pillay, SANAC Co-Chair Ms Steve Letsike, and two senior researchers from the HSRC, Prof. Geoffrey Setswe and Prof. Sibusiso Sifunda.

HEAIDS marketed its satellite session extensively ahead of the conference and attracted a sizeable audience drawn from its partners and stakeholders, as well as other interested individuals. Questions and discussion on the KAB Study results were insightful and stimulating, reflecting the extent to which participants had engaged with the findings.



1.8 Communication resources and activities

The Communication Programme at HEAIDS embraces two main areas of work:

- Media relations and stakeholder communication
- Production of information, education and communication (IEC) materials and marketing collateral for activations and other engagements with the student community.

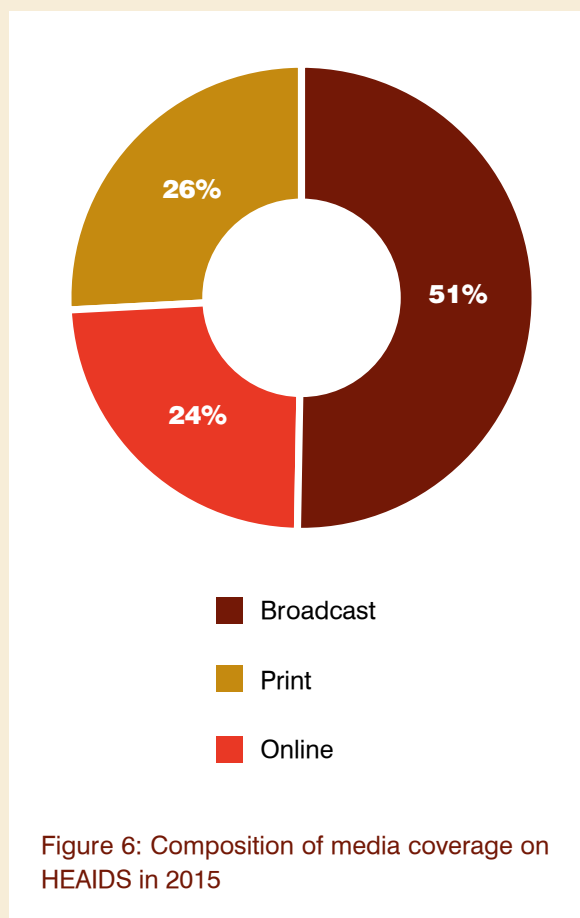
The public discourse on HIV, as reflected in the volume and nature of media coverage, has waned in recent years. The epidemic no longer commands the attention that it should, given the huge number of people who become infected each year, the significant number of deaths that persist in the face of accessible treatment, and the toll this continues to take on society and the resources of the country.

HEAIDS believes that media outreach can play a critical role in revitalising public awareness of the epidemic as a major national concern and that the higher education sector should be an opinion leader on this question. It is also in HEAIDS' interest to profile its achievements and advocate for the positions it supports. In particular, mass media platforms enable HEAIDS to reach parents of the 2 million students in the sector.

HEAIDS therefore adopts a proactive approach to media engagement and during the course of the year received 204 items of coverage on radio and TV, online and in the print media. This coverage was almost entirely fair and positive in its representation of HEAIDS. Its total value, at advertising rate equivalence, was R5.03 million.

Broadcast media accounted for the largest slice of coverage, mainly because radio stations tend to rebroadcast news items several times in a 24-hour cycle and each instance is counted as an item of coverage.

Stakeholder communication during 2015 occurred through the HEAIDS website, its social media platforms, a new format newsletter (introduced towards the end of the year) and various research and programme



publications, as well as through stakeholder participation in launches, events, conference satellite sessions and exhibitions.

HEAIDS undertakes the writing and production of IEC materials because access to accurate and relevant information is an essential element in HIV prevention and also drives uptake of available health services. Some institutions have done an outstanding job over the years in producing creative, exciting materials. But not all universities and colleges are equipped to do this and even the best-resourced may not be able to cover the entire range of topics embraced by HEAIDS programmes.

HEAIDS, therefore, produces materials ranging from branded marketing products to informative leaflets and posters. A commercial courier is utilised to make two major deliveries of materials to the main campus of every university and TVET college each year – at the start of the year and mid-year.

1.9 Technical support to institutions

The hidden hand that steers the activities outlined above is a relentless schedule of visits by HEAIDS managers and provincial co-ordinators to universities and colleges. These visits, together with regional workshops and meetings where clusters of institutions can share experiences and good practice, are the key to development and constant improvement of institutional HIV programmes.

Because there are a large number of TVET colleges spread across the country, a team comprising the Programme Manager for TVET Support and eight provincial co-ordinators was constituted to conduct regular visits. Collectively, members of this team conducted 580 visits to TVET college campuses during 2015. The University Programme involved 27 visits to universities during the course of 2015.

These visits serve to provide technical support for programme implementation, monitor progress, and link institutions with government, NGO and development partners that could play a role in strengthening their work.

Regional workshops for universities took place in all provinces except the Western Cape, while meetings of TVET colleges were held in all provinces. The latter

were especially valuable in resolving common teething problems experienced during this early phase of the HEAIDS Programme in the TVET sub-sector.

During the course of the year, various HEAIDS representatives, including the Director, attended meetings of provincial AIDS councils in the Eastern Cape, Free State, Gauteng, KwaZulu-Natal, North West and the Western Cape. These opportunities to interact with top officials from the Office of the Premier and a range of provincial government departments were invaluable in paving the way for on-the-ground partnerships with health facilities and other government services that benefit universities and colleges.

Partnerships between provincial and municipal health services and higher education institutions in their areas are particularly critical. Among other things, they secure the supply of medical consumables, such as test kits, medicines and condoms, to many institutions. They also underpin sustainable referral systems between universities and colleges, on the one hand, and local clinics and hospitals, on the other. Increasingly, campus health facilities are being treated as an intrinsic part of district health services and are resourced accordingly. There are instances where the district health office has gone as far as allocating professional staff to provide on-site health services to students and staff at TVET colleges.

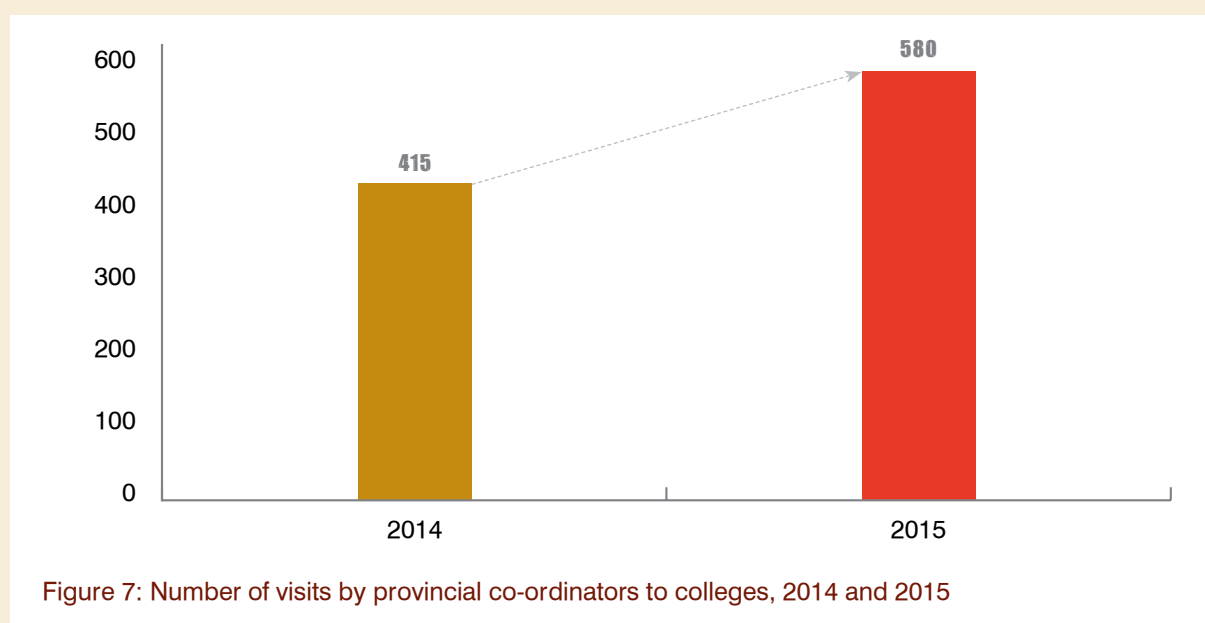


Figure 7: Number of visits by provincial co-ordinators to colleges, 2014 and 2015

1.9.1 Direct grants to universities

Both the Global Fund to Fight AIDS, TB and Malaria and the NSF made direct grants to universities during the course of 2015. In both instances, HEAIDS is responsible for managing these grants and ensuring that universities comply with all operational and reporting requirements of funders. The fact that these are performance-based grants is taken extremely seriously by HEAIDS, which conducts capacity-building workshops as part of its grant management process.

Some universities also received Global Fund grants via NACOSA for the development of the LGBTI Programme on their campuses.

Universities that received all three grants would have benefited by at least R1 million each.

HEAIDS encouraged an inclusive, co-ordinated approach to donor funding and avoided a situation where individual universities are bidding against each other.

In July 2015 Global Fund direct grants totalling more than R8 million were approved in respect of 23 universities – that is, all public universities operating at the time of application.

- Eighteen universities were allocated grants ranging from R350 000–R400 000
- The remaining five each received R200 000.

The purpose of these grants is to strengthen existing HIV programmes at campus level and enhance the ability of universities to contribute to the achievement of goals and objectives of the National Strategic Plan on HIV, TB and STIs.

More details on the Global Fund Direct Grant are contained in the Universities Programme section of this review.

The NSF direct funding to universities was for the purposes of HIV Curriculum Development. Each of the participating universities received approximately R450 000 which will be disbursed over a three-year period. The new universities missed out on the original grant, but two of them later submitted applications which were successful.

1.10 Summary of financial status

HEAIDS' income for 2015 amounted to R35.2 million and the programme's budget for the year matched this amount. The main sources of income were:

- The National Treasury allocation channelled through the DHET (22%)
- The NSF grant for curriculum development (38%)
- Membership contributions from public universities and colleges (3%)
- The Global Fund direct grant (35%)
- GIZ funding (2%).

As is evident from this breakdown, nearly two-thirds of HEAIDS' income for the year was derived from the national budget and channelled through the DHET, the NSF and public universities and colleges.

While international donor and development funding is deeply appreciated, it is prudent for HEAIDS to diversify its sources of income and reduce dependence on donors.

Actual expenditure for the year totalled R27.6 million, amounting to 79% of the budgeted figure. The budget surplus was largely due to under-expenditure against the NSF grant for curriculum development. Only 56% of the first year's allocation under the grant agreement was spent. This was due to the fact that a fair amount of preparatory work was required before implementation (and expenditure) gained pace. The situation is highly unlikely to recur in 2016.

The largest single item of expenditure for HEAIDS was programme implementation, which accounted for 45% of the total. Human resources and administrative costs together accounted for 32% of spending.

Since USAf holds all fiduciary responsibility for HEAIDS, the financial records of the programme are audited along with those of USAf.

A more detailed financial report is presented in Section 4.

1.11 Acknowledgments and appreciation

HEAIDS flourishes because of the combined efforts of thousands of individuals in diverse organisations. This makes it impossible to thank each person and organisation individually, but we would like to assure you that every contribution is greatly appreciated. We would especially like to acknowledge those whose every working hour is devoted to building the HIV response in universities and colleges.

At national and international level I would like to thank:

Minister of Higher Education and Training, Dr Blade Nzimande	For his leadership in the field of higher education and continued support of HEAIDS
Deputy Minister of Higher Education and Training, Mduduzi Manana	For his unswerving commitment and investment of time and energy as the programme's top-level champion
Minister of Health, Dr Aaron Motsoaledi	For setting the tone for genuine inter-sectoral collaboration and unlocking precious health resources for students at institutions
The Global Fund, the National Skills Fund and GIZ	For the funding that empowers HEAIDS and institutions to undertake this work, and the interest in the programme's progress
Members of the HEAIDS Advisory Board	For devoting a share of their busy lives to HEAIDS, offering wise advice and advancing the programme's interests
Universities South Africa and the South African College Principals Organisation	For their unqualified support and facilitating interaction with their members and, in the case of USAf, its fiduciary oversight over the programme
Senior officials in relevant sections of the Department of Higher Education and Training and the Department of Health	For their expertise and practical assistance in various areas of programme activity
National NGOs in the field of HIV prevention and care	For working by HEAIDS' side in a spirit of camaraderie and with a shared vision

At provincial and district level, HEAIDS' appreciation goes to:

MECs for Health and Mayors	For visible support and for facilitating the flow of key resources for campus healthcare
Provincial and district health officials and their staff	For building campus health and HIV interventions through resourcing and hands-on service provision
Provincial AIDS councils	For their interest in HEAIDS and facilitation of partnerships that have strengthened the programme
NGOs working in the province or district	For enthusiastic and reliable participation in FTF activations, support in campus capacity building, and provision of health services to referred students

At university and college level I would especially like to acknowledge:	
All rectors, principals and staff	For visible support to the programme and many actions that facilitate its smooth operation at institution level
HIV programme managers and communicators and campus health facility managers and staff	For keeping the heart of the HIV, TB and STIs programme of prevention, care and treatment beating through practical leadership, daily attention and problem-solving
HIV Institutional Co-ordinating Committees	For leadership and lobbying for the HEAIDS cause and guidance to programme implementation teams
Student leadership	Simply for being leaders and helping to build a health-seeking culture among students at institutions
Peer educators	For contributing to the building of a sustainable HIV, TB and STI programme and giving it an authentically young character
Academic staff participating in the programme	For taking the initiative in developing relevant HIV content for a variety of curricula and their willingness to share this with their colleagues
Campus and community radio stations in the Future Beats initiative	For your willingness to innovate and take risks in order to improve health awareness among your listeners

Finally, I would like to acknowledge my team in the HEAIDS National and Provincial Office. Much of their work is done behind the scenes, encouraging and enabling others to take action in implementing various aspects of the programme. This requires good judgment, maturity and well-honed people skills as well as technical management ability. I thank the team for the outstanding job they have done.



Ramneek Ahluwalia
HEAIDS Director

SECTION 2: HEAIDS PROGRAMME PERFORMANCE

2.1 Universities Programme

The Universities Programme provides support to 26 public universities in the planning and management of their HIV programmes. Most universities have been part of the HEAIDS initiative for about 15 years and have mature programmes, although some are constrained by limited resources. The establishment of Sol Plaatje University, Mpumalanga University and Sefako Makgatho University has presented HEAIDS with the unusual opportunity of assisting young institutions to initiate HIV programmes as an integral part of their development.

2.1.1 Campus activations

First Things First (FTF) activations encourage students to prioritise their health and to take responsibility for their wellbeing. They aim to build health awareness, prevent disease and detect both disease and risk factors at an early, manageable stage.

Each activation extends over a period of two to five days and involves various stakeholders on campus as well as external partners – including funders, government departments, service providers and advocacy groups. All activations focus on the prevention, early detection and treatment of HIV and other STIs as well as TB. Increasingly, FTF activations include education and screening related to diabetes, certain forms of cancer, and cardiovascular conditions. They also include the promotion of condoms and other forms of contraception. General education,

individual counselling, screening and testing, and referral for treatment all take place under the umbrella of activations.

The demand for activations grew substantially in 2015 and HEAIDS supported the implementation of 190 university activations – 44% more than in 2014.

Gauteng universities organised the largest number of activations, followed by those in KwaZulu-Natal and the Western Cape. It was also in these provinces that student protests created conditions unsuitable for activations and led to the cancellation of 11 activations.

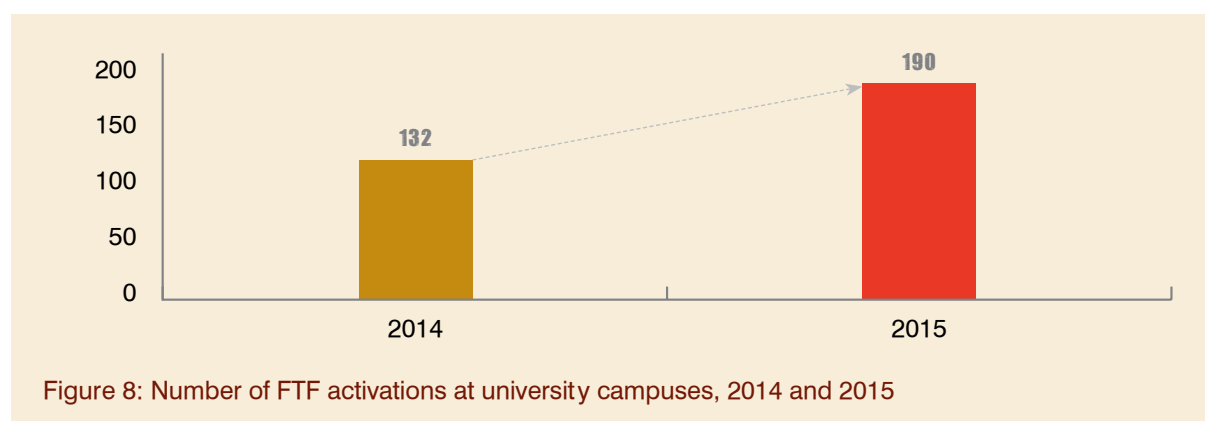


Figure 8: Number of FTF activations at university campuses, 2014 and 2015

2.1.2 HIV testing and other health screening

The great majority of students and staff who underwent HIV testing on campus in 2015 did so during activations held under the banner of the FTF Programme (Figure 9).

The success of these activations in 2015 is reflected in the 60% increase in HIV testing at universities (Figure 10). Similar increases can be seen in relation to TB screening and STI screening.

As in the wider population, females testing on campus outnumber males. Two out of every three individuals taking an HIV test at universities in 2015 were women.

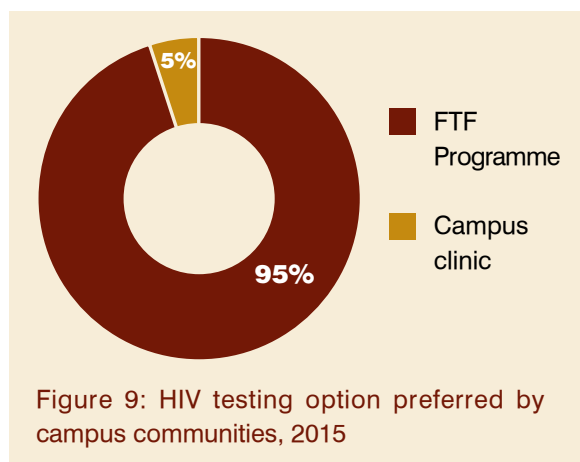


Figure 9: HIV testing option preferred by campus communities, 2015

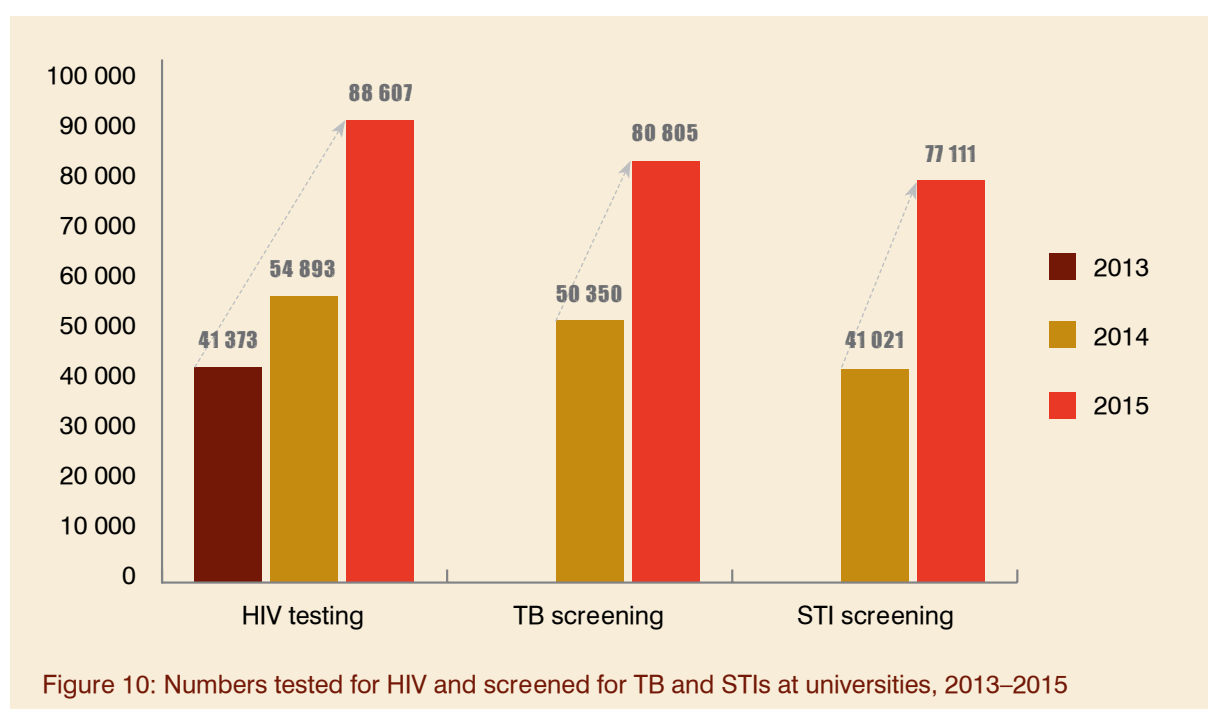


Figure 10: Numbers tested for HIV and screened for TB and STIs at universities, 2013–2015

2.1.3 Condom distribution

The Department of Health's decision to pilot the introduction of its coloured, scented condoms in the higher education sector resulted in a massive increase in the availability of state-funded condoms at universities. In just a year, the total number of male condoms distributed at universities alone rocketed from 114 642 to more than 20 million. Several factors contributed to the system's capacity to deal with this increase, including the involvement of the Society for Family Health as a bulk distributor and organiser of

major condom promotion campaigns, and Global Fund direct grants, which enhanced capacity on campus to get condoms into the hands of those needing them.

The relative scarcity of female condoms is a standing issue across South Africa and the higher education sector is no exception. Against this background, the 239% increase in the supply of female condoms on university campuses during 2015 was encouraging.

Table 2: Number of condoms distributed at universities, 2014 and 2015

Type of condom	2014	2015
Male condoms	114 642	20 179 085
Female condoms	17 738	42 339

2.1.4 Institutional support visits and regional workshops

The HEAIDS Manager for Universities Support is responsible for visiting all public universities at least once a year to provide technical advice on the conduct of their HIV programmes and to monitor progress. During 2015, 27 such visits took place involving 23 universities. Consultations took place telephonically with two of the three universities not visited (University of Pretoria and University of Zululand), while representatives of the newly established Mpumalanga University visited the HEAIDS office.

This individualised attention assists managers of institutional HIV programmes in enhancing their programmes under the unique circumstances that they experience. Questions of comprehensive programming, integration with other campus health and wellness initiatives, and sustainability are central to discussions. In addition, HEAIDS has relationships with more than 300 structures in the development, public health and NGO sectors and seeks to facilitate co-operative relationships between relevant organisations and individual universities.

In 2015, technical support visits also prepared universities for the introduction of the HEAIDS Integrated Online Reporting System (HIORS) and supported the internal audit of funds disbursed under the Global Fund direct grants.

Five regional HEAIDS workshops were convened during the year, providing forums for universities to learn from each other and engage with key partners in government, the international development arena, and the NGO sector. A specific aim during 2015 was to begin building relationships between universities and TVET colleges operating in the same region.

Other important events during the year included:

- The HIV Policy Development Workshop for New Universities, which was convened for the benefit of Sol Plaatje University, Mpumalanga University and Sefako Makgatho University
- The Unisa Regional Co-ordinators' Workshop, which was designed to enable the institution's regional co-ordinators to better assist Unisa campuses in implementing activations
- Training sessions on the new HEAIDS Integrated Online Reporting System
- The inaugural meeting of the HEAIDS National Forum for University HIV Managers, which will serve as a platform for managers to share best practice, discuss policy issues, and address common concerns
- The HEAIDS Campus Health Meeting, which afforded heads of campus health services the opportunity to discuss their interface with HIV programmes at their institutions and to engage the Department of Health to develop joint solutions to resourcing and sustainability challenges
- The HEAIDS Employee Health and Wellness Meeting, which focused the various health-related stakeholders on campuses on the needs of workers in the university sub-sector. This forum will meet twice a year to address this somewhat neglected area of programming.

2.1.5 Global Fund direct grant

In July 2015 Global Fund direct grants totalling more than R8 million were approved in respect of 23 universities and monitoring systems were put in place to ensure appropriate utilisation of funding.

The purpose of these grants is to strengthen existing HIV programmes at campus level and enhance the ability of universities to contribute to the achievement of goals and objectives of the National Strategic Plan on HIV, TB and STIs. These grants generally enabled universities to participate more actively in the main campaigns conceptualised and co-ordinated by HEAIDS. Details of the allocations appear in Table 3.

Table 3: Allocations to universities from the Global Fund direct grant

Granted R200 000
University of Cape Town
University of KwaZulu-Natal
University of the Western Cape
University of Venda
University of Zululand
Granted R350 000
Cape Peninsula University of Technology
Central University of Technology
Walter Sisulu University
Granted R400 000
Durban University of Technology
Mangosuthu University of Technology
Nelson Mandela Metropolitan University
North-West University
Rhodes University
Stellenbosch University
Tshwane University of South Africa
University of Fort Hare
University of Johannesburg
University of Limpopo
University of Pretoria
University of South Africa
University of the Free State
University of the Witwatersrand
Vaal University of Technology

HEAIDS closely monitors the utilisation of these grants and when a university had failed to spend any of the allocation several months into the grant cycle, a decision was made to withdraw the allocation and distribute the funds to other institutions.

2.1.6 Building capacity for peer education

Peer educators work at the coal face of health education and mobilisation in HIV, TB and STI-related campaigns. They require sound knowledge of the basic facts regarding all health areas covered by HEAIDS campaigns, and an especially good understanding of prevention methods and risk mitigation. In addition to this, they need to embody and promote attitudes that help campus communities deal with stigma and relevant human rights issues.

Over the years, the training of peer educators on campuses has increased (Figure 11). Given the nature of the sector, with its constant turnover of students, the task of peer educator training will never be done. There will be a constant need to recruit peer educators from new generations of students and provide them with relevant training. As new emphasis and areas of work are added to the HEAIDS Programme, it is also necessary to provide supplementary training to experienced peer educators.

In 2015, HEAIDS enabled universities to train about four times the number of peer educators that were trained just two years earlier. This upward trajectory will be maintained into 2016, which will hopefully see some 2 000 peer educators trained in the university sub-sector.



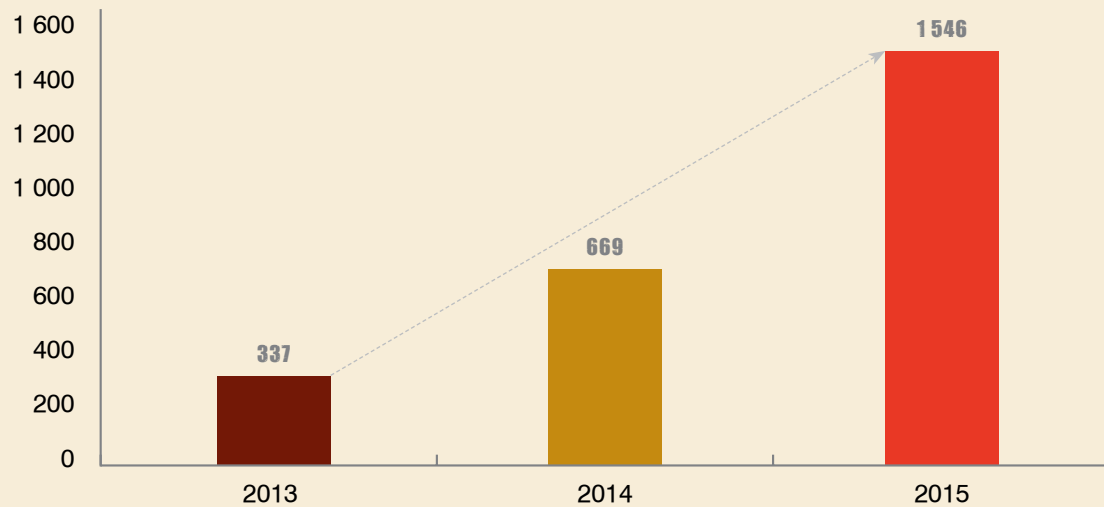
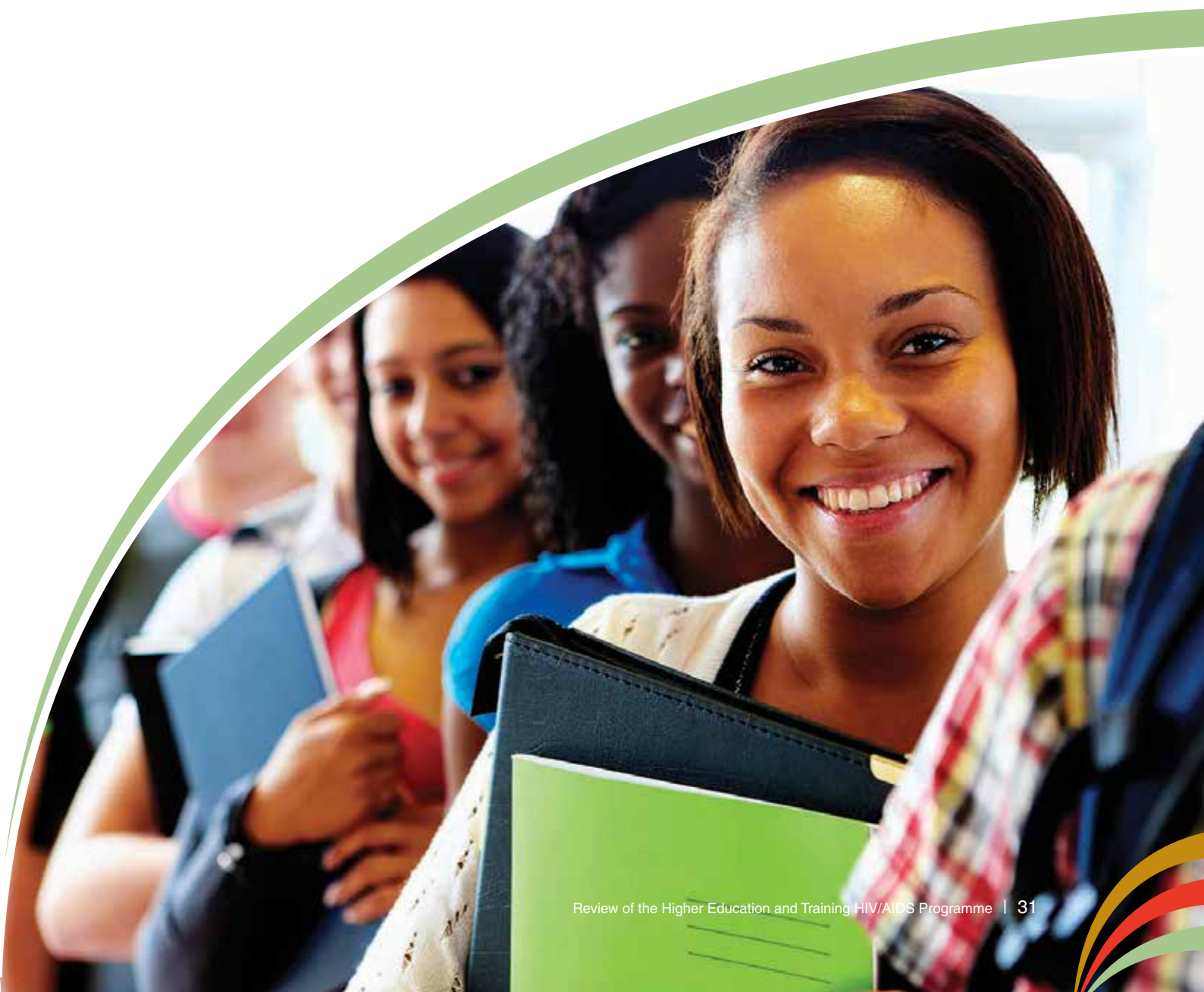


Figure 11: Number of peer educators trained at universities, 2013–2015



UNIVERSITIES IN THE LEAD

HEAIDS senior management attended, and often participated in, significant events initiated and organised by universities. Some of these are briefly described below.

Women's Health Empowerment launch

University of KwaZulu-Natal (UKZN)

The university linked its Women's Health Programme with the KwaZulu-Natal Health Department's campaign on dual protection against pregnancy and HIV. The launch enabled the university to showcase its successful peer education approach and campus women's clubs. It was addressed by the MEC for Health, Dr Sibongiseni Dhlomo, and attended by more than 300 students, provincial and district HIV officials, and a range of internal university stakeholders. The participation of TVET colleges signalled a commitment by the university to work more closely with colleges in future.

Alcohol and Drug Abuse Prevention Programme

Tshwane University of Technology (TUT)

TUT utilised its Global Fund direct grant to conduct a multi-campus campaign, Lives on Board, to prevent alcohol and drug abuse among students. The launch featured Vice-Chancellor, Professor Lourens van Staden, and Project Officer, Sithembiso Magagula, the dynamic force behind the Alcohol and Drug Abuse Prevention Programme. It drew about 200 students as well as representatives from partner organisations, such as the district and provincial health services and the South African National Council on Drug and Alcohol Abuse.

Student Health and Wellness Strategy Workshop

University of South Africa (Unisa)

Because all members of Unisa's enormous student body pursue their studies by distance learning, the university faces particular challenges in offering an effective health and wellness programme to its students. During 2015, Unisa's Student Development Department convened a workshop to develop a health and wellness strategy designed to serve thousands of students, even in the remotest parts of South Africa and beyond the country's borders.

Sexuality Seminar

University of Cape Town (UCT)

UCT pooled the knowledge and experience of researchers, lecturers, LGBTI students and staff members, HIV project officers, counsellors and student leaders to answer the question: what would a comprehensive HIV response for the LGBTI community on campus entail? There was considerable agreement on the nature of an "ideal" programme and a shared conviction that the programme must address broader social, cultural and psychological factors. Too narrow a focus on HIV itself would not achieve the desired health outcomes.

Western Cape Youth Leadership HIV and AIDS Conference **Cape Peninsula University of Technology (CPUT)**

The conference was attended by student and management representatives from a range of universities and colleges in the region as well as non-governmental organisations. Held under the slogan “Make your mark by leading the change”, it was noteworthy for its engagement with difficult issues, including alcohol and substance abuse and inclusion of LGBTI in programme work. It also achieved a new level of interaction on HIV between universities and TVET colleges in the region.

Soweto launch of global ProTEST Campaign **University of Johannesburg (UJ)**

UNAIDS selected HEAIDS as its local partner for the global campaign on youth testing, ProTEST, and UJ’s Soweto campus hosted the campaign launch, which took place on Nelson Mandela Day. Three UNAIDS youth ambassadors and musicians from the United States and Norway entertained the crowd, while the serious business of testing proceeded. By the end of the event, 204 students had undergone testing, 58 of them for the first time.

Opening of new health clinic **University of Venda for Science and Technology (Univen)**

The university’s capacity to service the health needs of students and staff received a huge boost with the opening of a new health clinic, capable of providing comprehensive primary healthcare, including family planning and prevention and treatment of HIV, STIs and TB. This new facility will eliminate a major obstacle to serving the health needs of students and staff. The well-attended opening ceremony was headed up by Limpopo MEC for Health, Dr Phophi Ramathuba, Vice-Chancellor Professor Peter Mbatlali, and members of council, and the occasion doubled as lift-off for the purple grape-scented condoms.

2.2 TVET College Programme

This programme experienced tremendous expansion during 2015, which was the second year that TVET colleges formed part of the HEAIDS initiative. Behind this expansion in HIV interventions were a busy schedule of meetings with individual colleges and local stakeholders and a series of capacity building activities.

It was important to recognise the similarities and differences between TVET colleges and universities during the development of the TVET HIV, TB and STI Programme. The fundamental similarity lies in the fact that both types of institution offer programmes of learning to young people who are at a stage of life when they are particularly at risk of HIV infection.

The differences to be taken into account when enabling TVET colleges to develop HIV/AIDS and general wellness services for students and staff are:

- Individual college campuses are much smaller than most university campuses. They do not have dedicated health clinics and health personnel on site and enrolment would often not warrant this
- There are many more TVET colleges than universities and they often have multiple campuses.
- Collectively TVET colleges have a presence in every district of South Africa
- TVET colleges tend to be more integrated into communities, drawing their students from the surrounding area. Generally they do not have large numbers of residential students
- TVET curricula are strongly geared to specific occupational requirements and more practical in nature than most university courses
- Research is not a prominent feature of the TVET college mandate, in contrast to universities.

HEAIDS' strategy in marshalling resources to support HIV and related activities on TVET campuses has been to capitalise on the fact that they are strongly embedded in communities and to draw on resources from these areas to benefit the colleges. Another strategic consideration has been the employment by HEAIDS of provincial co-ordinators to provide technical support and assist with organisation of activations until such time as capacity on college campuses has been strengthened.

A positive feature of the limited size of TVET campuses was that it was generally possible to engage directly with the head of the institution and obtained this high-level interest in the development of HIV, TB and wellness interventions. In a similar way, it was easier for campaigns and activations to have campus-wide impact and reach a high percentage of students.

2.2.1 Campus FTF activations

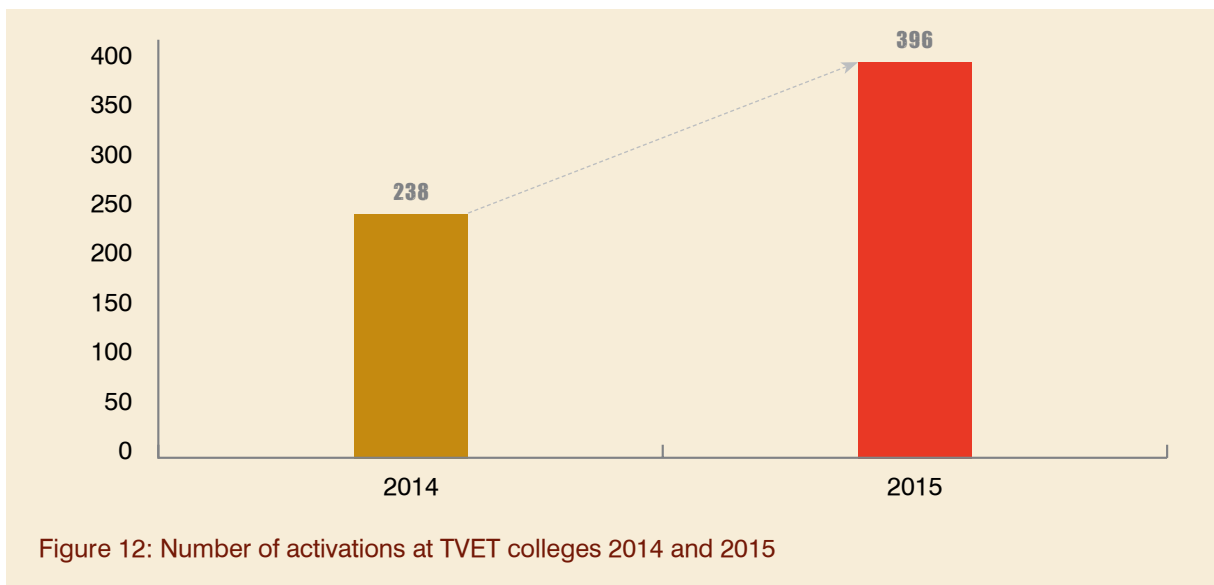
During 2015, the number of activations conducted at TVET colleges increased 66% relative to the previous year. The distribution of activations by province was roughly in proportion with the size of the TVET student population in the province.

The activations follow the same model as those at universities – health awareness activities combined with health screening – and are branded as the HEAIDS First Things First Campaign.

A number of activations (83 or 17% of the total number requested) did not take place as intended. In two-thirds of cases this was due to student protest action, while the remainder of cancellations were due to co-ordination shortcomings, unavailability of key partners, and closures of colleges for reasons other than protests.

Table 4: TVET college activations undertaken during 2015 by province

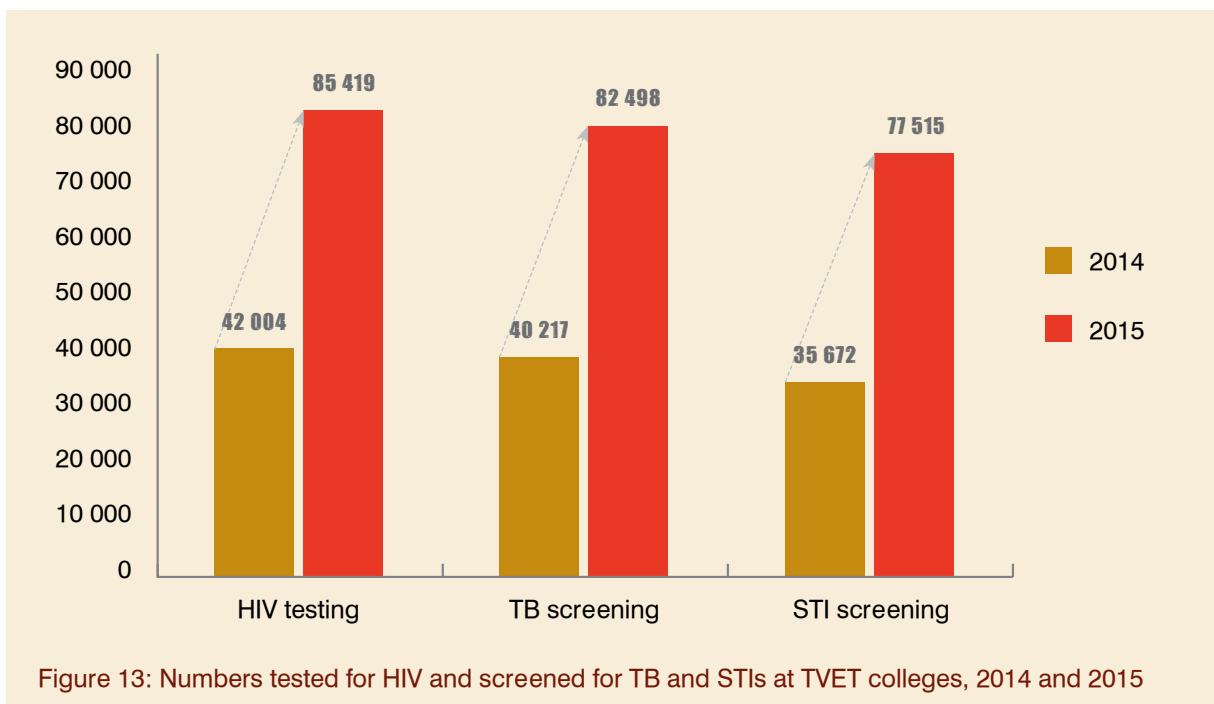
Province	Activations delivered
Eastern Cape	58
Free State	35
Gauteng	83
KwaZulu-Natal	52
Limpopo	35
Mpumalanga	37
Northern Cape	12
North West	22
Western Cape	62
Total	396



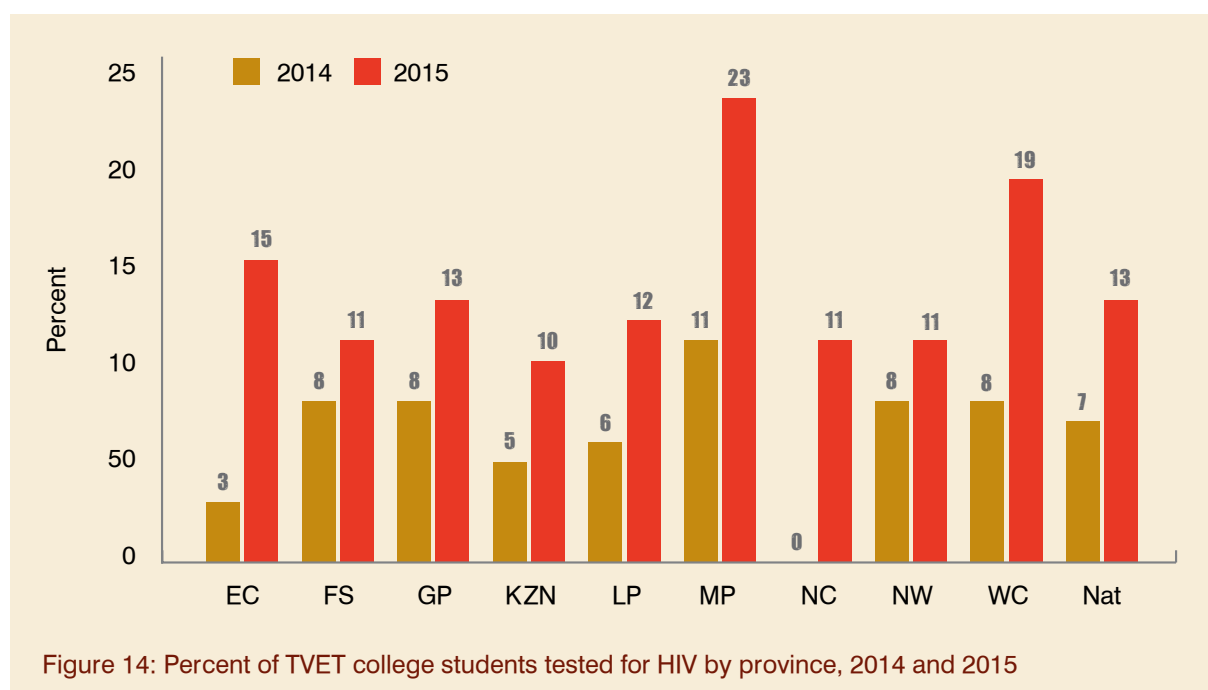
2.2.2 HIV testing and other health screening

An impressive increase in the number of students and staff testing for HIV and undergoing TB and STI screening accompanied the rise in the number of activations.

With a total of 85 419 HIV tests recorded in 2015, the previous year's total was more than doubled. There was strong demand for testing and screening throughout the year. The familiar pattern of women outnumbering men was detectable in the TVET sector, where 66% of those testing were female.



Coverage of HIV testing across the TVET student population nationwide reached an impressive 19% in 2015 and did not fall below 10% in any province. Once again it should be noted that provinces with the largest student populations tended to attain slightly lower than average coverage, but the magnitude of their task must be taken into account. The enormous improvement in coverage in the Eastern Cape (from 3% in 2014 to 15% in 2015) was especially impressive, as was Mpumalanga's ability to test nearly one in four students.



2.2.3 Condom distribution

Systems for condom distribution at TVET colleges only became operational from May 2015. This development coincided with the government's piloting of the new grape-coloured and -scented male condom so the scale-up was rapid. In a matter of months, TVET colleges were receiving and distributing quantities that greatly exceeded what universities were able to manage at a similar stage of development.

In seven months, a total of 4.7 million scented, coloured condoms were distributed at colleges in six provinces (see Table 5).

Table 5: Number of government condoms distributed at colleges by province, 2015

Province	Number of condoms distributed
Eastern Cape	1 080 000
Free State	1 692 000
Gauteng	1 140 000
KwaZulu-Natal	642 000
Limpopo	Stock unavailable
Mpumalanga	Stock unavailable
Northern Cape	Distributing own condoms
North West	120 000
Western Cape	48 000
Total	4 722 000

2.2.4 Peer educators

Peer educators play a key role in sustaining health awareness activities on campuses. They not only participate in major activations, but ensure that condom distribution continues throughout the year alongside the conduct of health dialogues and group discussions. The peer education system was not yet fully functional in the TVET sub-sector in 2015, but a strong foundation was laid.

As in the university sub-sector, peer educator training covers broad information on HIV, TB and STIs and subjects that are key to the Women's Health Empowerment Programme, Men's Health Empowerment Programme, LGBTI Programme and Alcohol and Drug Abuse Prevention Programme.

In 2015, a total of 3 349 peer educators were trained and the 124% increase on the previous year's number (1 493) reflected enormous commitment by all role players. By training staff members as well as students under the peer education programme, it is possible to utilise staff members as mentors and to prepare the way for improved HIV, TB and other health content in the Life Orientation curriculum.

In 2015, the demand from TVET colleges for peer educator training began to outstrip the capacity of HEAIDS' existing delivery mechanisms. For two years HEAIDS has used skilled and experienced

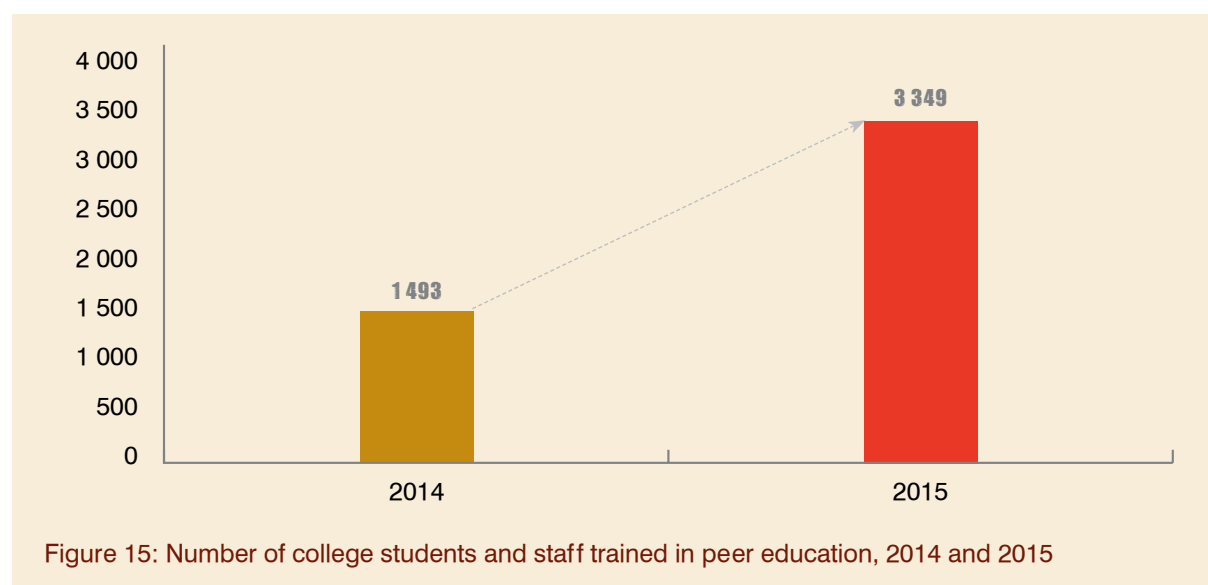
partner organisations, such as the Foundation for Professional Development (FPD) and the Centre for Communication Impact (CCI – previously Johns Hopkins Health and Education in South Africa), to undertake peer educator training.

HEAIDS intends to establish an in-house Peer Education Programme in 2016 that will rest on the upskilling of its provincial co-ordinators and selected "peer education mentors" in the college environment. Through a train-the-trainer initiative, these individuals, who already have a sound understanding of HIV, TB and STIs, will be equipped with the skills needed to train peer educators.

2.2.5 The provincial co-ordination team

The backbone of the TVET Colleges Programme is the team of eight provincial co-ordinators who work under the guidance of the HEAIDS Programme Manager for TVET College Support to steer the energies of committed managers, staff members, student leaders and peer educators at colleges.

The establishment of college-specific HIV Institutional Co-ordinating Committees (HICCs) is still work in progress in the TVET sub-sector. These committees have played a critical role in the university sub-sector and HEAIDS is confident that they will be invaluable in ensuring that TVET colleges take ownership of their HIV/AIDS programmes in the period ahead.



2.2.6 Institutional visits and stakeholder liaison

The dedication of the TVET support team is reflected in the fact that they undertook a total of 580 visits to institutions in 2015, improving their 2014 performance by 165 visits.

Co-ordinators' challenges vary from province to province – for some, the sheer number of colleges presents a challenge, while others are faced with vast distances between colleges and a scarcity of community resources to provide support to campuses. The distribution of visits by province is shown in Table 6.

Table 6: Number of visits done by provincial co-ordinators by province, 2015

Province	Number of visits
Eastern Cape	79
Free State	41
Gauteng	137
KwaZulu-Natal	115
Limpopo	51
Mpumalanga	14
Northern Cape	5
North West	33
Western Cape	105
Total	580

The purpose of HEAIDS visits to colleges at this stage of programme development is to:

- Provide technical support and advice to colleges about the development of HIV prevention and health promotion programmes and health services for the benefit of students and staff
- Introduce colleges to external organisations (where necessary) and promote collaboration with the organisations so that they become partners in the development of college HIV programmes
- Facilitate problem-solving by colleges to overcome obstacles to programme implementation
- Monitor the progress that colleges are making in the delivery of HIV and health programmes
- Advocate and lobby within the college, where necessary, for the implementation of HIV and health programmes.

Often college principals attended these meetings in 2015 along with selected managers and staff (for example, student support managers and student liaison officers), student representative council members and peer educators. External participants included HIV and TB programme managers from district and provincial health services, members of provincial and district AIDS councils, and non-governmental organisations operating in the area (for example, New Start, Right to Care and SANTA).

2.2.7 Provincial meetings of TVET colleges

In addition to visiting individual colleges several times in 2015, HEAIDS also convened nine provincial meetings between January and September. The goal was to create a forum for TVET colleges from each province and their funding and implementation partners to discuss common concerns, potential solutions and best practice in the implementation of the HEAIDS Programme. In addition to the TVET College Programme staff, the HEAIDS Director and senior managers attended these meetings, along with a senior manager of the National Skills Fund. Representation from the TVET colleges was excellent at most meetings.

The hectic schedule of visits during 2015 and the provincial meetings yielded a variety of fruits:

- There was increased buy-in for the HEAIDS Programme at colleges as it became more real and its advantages apparent
- The sub-sector delivered a large number of activations that were generally well-organised. This was largely due to provincial co-ordinators working side-by-side with colleges on implementation, bringing their experience to bear
- Progress was made towards the establishment of HICCs at college level
- Interaction with local external stakeholders began to translate into partnerships that gave some TVETs access to added resources and services. For example, the Limpopo Health Department allocated eight full-time nurses to provide services at five TVET colleges, the eThekweni Health Department offered training for college staff members, and the Family and Marriage Society of SA (FAMSA) started a support group for HIV-positive students at a Free State college. These are just a few examples, and although the partnership route still needs a lot of

exploring, they illustrate the degree to which the sharing of resources can make a difference in students' lives.

2.3 HIV Curriculum Development Programme

HEAIDS strives to build on the higher education sector's core mandates of teaching and learning, and research and innovation in order to equip graduates to manage the impact of these epidemics in their personal and professional lives. In this way, HEAIDS contributes to the higher education goal of supplying suitably equipped graduates to the job market and at the same time helps build the national HIV, TB and STI response.

Recent activities undertaken under the Curriculum Programme have been informed by various studies HEAIDS commissioned to understand how HIV could best be addressed in the academic space. A finding that emerged clearly from this body of work was that while academics appreciated the rationale for HIV content in curricula and were willing to play a part, they articulated a need for support. They wanted resources and they wanted capacity-building opportunities for themselves.

In 2015 HEAIDS succeeded in securing support from the National Skills Fund (NSF) and was awarded a three-year discretionary grant to develop the capacity of academic staff at universities and colleges to integrate content on HIV into their curricula, both in a generic way and within the focus areas of different disciplines.

Universities and colleges differ quite significantly in terms of the environments they present and the

opportunities that exist for curriculum integration. Therefore HEAIDS has adopted different approaches to the utilisation of the NSF grant in the two sub-sectors.

2.3.1 Universities and HIV curriculum development

The modality in the university sub-sector took the form of direct grants to 20 universities. These institutions each received a three-year allocation of about R450 000 to explore how HIV and related issues could be incorporated into their curricula. The approach took account of the university culture of institutional autonomy and was based on the premise that academics were best placed to design interventions to contribute to the body of knowledge in this area.

The universities that received these grants were Cape Peninsula University of Technology, Central University of Technology, Durban University of Technology, Mangosuthu University of Technology, Nelson Mandela Metropolitan University, North-West University, Rhodes University, Tshwane University of Technology, University of Cape Town, University of Fort Hare, University of Johannesburg, University of Limpopo, University of Pretoria, University of South Africa, University of the Western Cape, University of the Witwatersrand, University of Venda for Science and Technology, Walter Sisulu University, University of Mpumalanga and Sefako Makgatho Health Science University.

A key component of the programme was capacity development of academics and a model was developed to provide focus and direction to this process. Through dialogue and engagement with academics, several thematic areas were identified and shaped into development workshops.



Table 7: Thematic areas for developing and delivering HIV content in university curricula

Theme	Description
Models of curriculum integration	The aim of these workshops is to enable participating academics to select approaches to HIV content integration relevant to their institutional context. Workshops explore different approaches to curriculum integration and offer innovative ways to include HIV across modules and within disciplines. These make the links to students' own lives and the world of work.
Critical diversity literacy	The aim of workshops is to engage academic staff in examining how factors such as race, sexual identity, gender, social and economic status, and the exercise of various types of power impact on the course of HIV. Participants are encouraged to examine their own assumptions, beliefs and biases in order to enable them to be more sensitive to the differing needs of a range of students when dealing with HIV.
Participatory pedagogy	The purpose of workshops is to enable academics to understand models and methods of participatory pedagogy. The workshop actually demonstrates different methodologies and explores the key notions of "curricula in the making" and "humanising pedagogy".
Institutional conversations on emerging good practice	The workshop focuses on teaching approaches that encourage learners to engage with HIV issues as they might arise in the workplace. Content includes labour legislation and workplace discrimination. Participants present innovative techniques for integrating this information into their teaching. The emphasis is on networking and sharing among participants.
HIV 101	The purpose of the workshop is to ensure that academics have up to date information on developments in the field of HIV and related matters. It provides basic statistics and epidemiological trends, information on prevention and treatment approaches, and deals with relevant social and human rights issues. This information can be used in discipline-specific curricula to help prepare students for the workplace challenges HIV may pose in their chosen occupations.

During the course of 2015, a total of ten workshops took place and 402 academics in a range of disciplines participated in these.

Table 8: Faculties and disciplines represented at training workshops on different themes

Models of Integration Workshops	Institutional Dialogue: Curriculum Integration	M&E Workshops
• Health • Science and Agriculture • Environmental Engineering • Natural Science • Economics & Management • Arts & Design • Public Law • Technology • Education • Social Sciences • Education • Psychology	• Engineering & Built Environment • Informatics & Technology; Design • Health Sciences • Health & Wellness • Business Management • Science • Agriculture • Education • Humanities • Law • Management • Environmental Sciences	• Health • Science & Agriculture • Environmental Sciences • Engineering • Natural Science • Biochemistry • Economics & Management • Arts & Design • Law • Education • Office Management • IT Management • Accounting • Social Sciences • Education • Psychology
HIV 101 Training Sessions	Participatory Pedagogy Workshops	Critical Diversity Literacy Workshops
• Language and Literacy • Health Sciences • Agriculture • Farm Management & Plant Production • Engineering & Built Environment	• Health Sciences • Emergency Medicine • Arts & Graphic Design • Nursing Sciences	• Language & Literacy • Health Sciences • Agriculture • Farm Management & Plant Production • Teaching & Learning Unit
	Writing Retreat • Environmental Sciences • Biomedical Sciences	

Central to the delivery of this work are monitoring and documenting the development of a body of knowledge. Therefore the outputs and impact of these interventions are being monitored, with all universities contributing to the development of a single monitoring and evaluation framework.



2.3.2 TVET colleges and curriculum development

The NSF grant in respect of the TVET sector is designed to benefit all 50 colleges. It does not take the form of allocations to individual colleges, but enables HEAIDS to engage in a broader set of interventions that embrace the sub-sector. This approach accords with priorities of the Ministry of Higher Education and Training in respect of curriculum development. The National Curriculum and Development Unit at the DHET plays a key role in HIV curriculum development.

During 2015, a baseline research study was completed to establish gaps and opportunities that exist in college curricula and teaching capacity in terms of educating students on HIV and related matters.

The initial findings of this baseline study were circulated to TVET colleges as well as key stakeholders and, with the assistance of a special advisory committee that met in September, three initial work streams were defined: Life Orientation, Agriculture and Mining. Curriculum strengthening and capacity development will take place in these three areas across the TVET sub-sector in 2016.

The **Life Orientation** Work Stream will focus on the 19 national certificate vocational (NCV) programmes that already contain a compulsory life orientation module. The baseline study established that there was a lack of standardisation in the practical element of life orientation modules. Through the NSF project, HEAIDS will support the development of a standardised practical assessment for life orientation. Practical performance in life orientation will be linked to the expansion of college peer education capacity, which in turn supports the delivery of HEAIDS FTF activations as well as ongoing health promotion at colleges. The intention is to standardise and strengthen practical learning and at the same time provide colleges with a sustainable source of peer educators to provide services on campus and in surrounding communities. Lecturer capacity development will be an essential part of this work stream.

The **Agriculture** Work Stream will involve the revision of the current agriculture curriculum in collaboration with the DHET Curriculum Development Support Unit. This work will be undertaken through an international collaboration with McGill University in Canada and Ethiopia's vocational colleges, which have succeeded in mainstreaming HIV, gender and related issues into agricultural training and practice. The collaboration will be facilitated with the Ethiopian Department of Foreign Affairs, Trade and Development and the new curriculum will be piloted in KwaZulu-Natal.

The **Mining** Work Stream will seek to develop a special module on managing HIV, TB and related matters in the world of work and will be piloted in the NATED 191 programmes, which are currently devoid of HIV content. Technical guidance will be provided by the Chamber of Mines, Department of Mineral Resources, International Labour Organization and other relevant organisations. Five colleges based in the heart of mining communities will pilot the new module.

2.3.3 Successes and challenges

By and large, programme implementation has gained momentum and there has been significant progress in various spheres of HIV curriculum development. However, it is critical to appreciate that this is a challenging area of work as universities have to juggle many competing priorities. There is therefore a need to achieve buy-in to the programme by academics and this will require constant and innovative attention.

The definition of work streams in the TVET sub-sector was a welcome achievement and the support of TVET leadership in reaching this point was invaluable. However, systemic and capacity issues remain and will need to be carefully managed as the implementation gets under way. Changes within the TVET college environment, which commenced in April 2015, may complicate the process of HIV curriculum development and strategic use of leadership support will be important to ensure progress.

2.4 Future Beats Youth Development Programme

Future Beats is a youth development and HIV prevention programme that aims to develop radio and social media programmes for campus and community radio stations in order to increase awareness of HIV and related social and human rights issues among young people.

The project empowers programme managers and volunteer journalists at campus and community radio stations to produce interesting, accessible content on the issues mentioned above and engage in dialogue with their audience. It is funded by the German Development Agency, GIZ.

The project began in 2014 as a pilot initiative involving six radio stations in Gauteng, Limpopo and North West. A formal evaluation indicated the pilot project not only had an impact in terms of upskilling radio staff and focusing them on HIV-related issues, but was also associated with an increase in HIV testing on three campuses involved in the project.

In 2015, the Future Beats team at HEAIDS continued to work with the original stations, while adding a further five radio stations in Mpumalanga and the Eastern Cape to the project. This expansion saw the estimated audience reached by the Future Beats initiative grow from 270 000 in 2014 to 770 000.

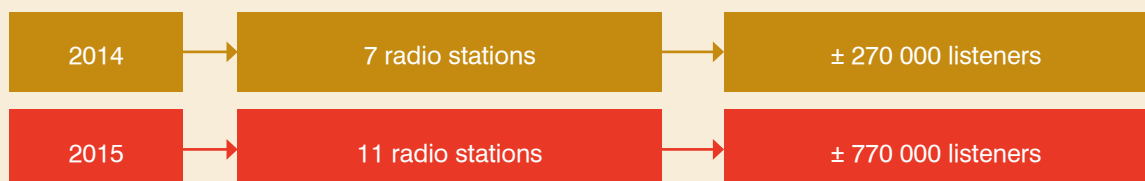


Figure 16: Growth in Future Beats Youth Development Programme

2.4.1 Capacity development

A total of 122 campus and community radio station journalists benefitted from a series of orientation and training workshops held during the course of 2015. These are summarised in the table below.

Table 9: Summary of Future Beats workshops/training sessions held in 2015

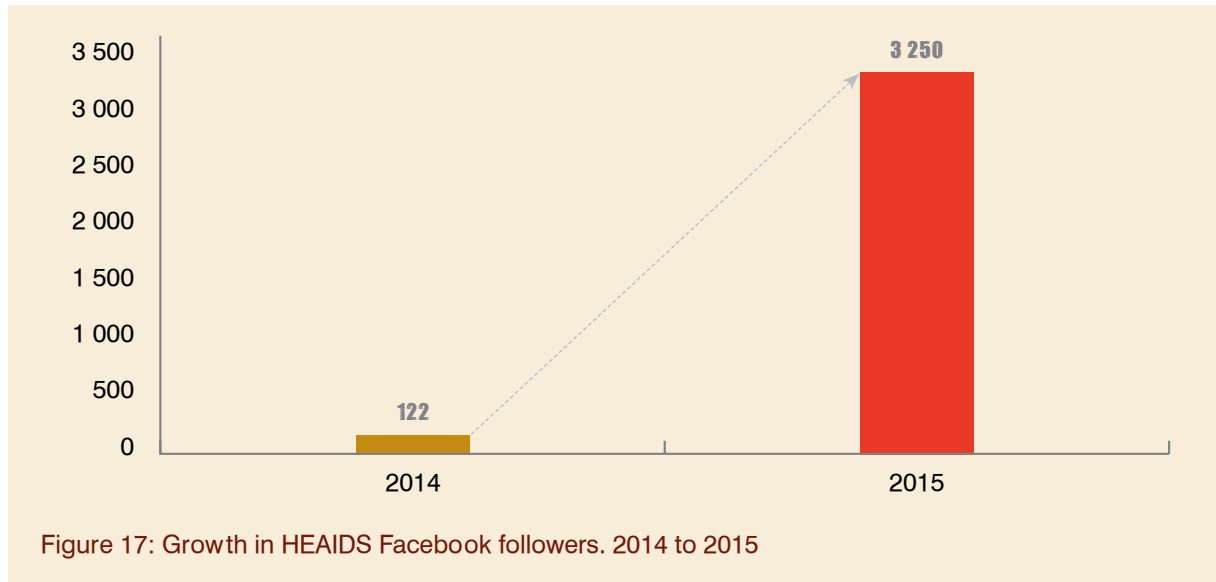
Type of workshop or training session	Number held and length	Participants	Partner/guest presenter
Planning and networking workshop to introduce new stations to Future Beats	One three-day workshop	New radio stations	Journalism lecturer, Eva Georgia
Sensitisation workshops to highlight social issues that contribute to spread of HIV	Two two-day workshops	New radio stations	CEO of Rebranding HIV, Phindile Sithole-Spong
In-house training on journalism techniques for production of high quality content on HIV and related issues	Five three-day training sessions	New radio stations	Executive Producer of Wits Radio Academy, Xoli Matomela
Facilitation training for presenters, focusing on techniques for sensitive topics. Designed to support stigma reduction	One one-day training session	Original radio stations	Radio producer and facilitation trainer, Lesley Nkosi
Radio drama training to provide insight into use of drama in HIV prevention	One seven-day training session	Original radio stations	Refiloe Lepere of Drama for Life at Wits University and Julia Ann Malone of SAfm

A formal evaluation of the Future Beats project was undertaken and a summary of the results appears in the Research Projects section of this review.



2.4.2 HEAIDS social media platforms

Future Beats has benefited HEAIDS more generally by contributing to the development of social media platforms. During 2015, HEAIDS had a presence on Facebook, Twitter, YouTube and Soundcloud and its Facebook followers totalled more than 3 000 by the end of the year.



SECTION 3: RESEARCH PROJECTS

HEAIDS has established a firm track-record of undertaking research to produce the evidence for programme planning and implementation. Two major studies and an evaluation of the Future Beats Programme were published during 2015, while a third study was completed during the year and made available for internal consumption pending publication in 2016.



The **National Student Sexual Health and HIV KAB Survey Focusing on Student Men who Have Sex with Men**

was completed in 2014 and launched at a special event held at Unisa in 2015. It was funded by the Global Fund, managed jointly by HEAIDS and NACOSA, and undertaken by a team at Stellenbosch University.

The aim of the study was to inform the development of a comprehensive HIV programme for LGBTI students.

However, the insights gained will not only shape the HEAIDS LGBTI Programme, but also impact on HIV risk-reduction interventions for the entire student population. One of the main findings of the survey was that sexual identity and sexual practice among students cannot simply be equated. The situation is more fluid and complex – 30% of male participants who had sex with men did not view themselves as homosexual, but described themselves as bisexual or heterosexual. Half the male respondents who had sex with men said they had had at least one female sex partner in the last year.

A total of 8 869 students participated in the survey. Of these, 896 were men who had sex with men (MSM) and 7 973 constituted a male and female non-MSM group of respondents. Due to the difficulty in reaching MSM, the sample was not randomly selected and extrapolation to the student population as a whole is technically not possible. Despite this limitation, the survey advanced HEAIDS' understanding of the sexual health risks and needs of MSM in the student population.

Overall, respondents, including MSM, had a good understanding of HIV and its transmission, reported high levels of HIV testing (75% among MSM), and appeared to experience high self-esteem.

Sexual and social practices that increase the risk of HIV infection were reported by respondents in both the MSM and non-MSM groups. In the MSM group specifically the following behaviours were identified as potentially risky:

- Unprotected anal and/or vaginal sex
- Absence of lubricant use during anal sex
- Multiple concurrent sex partners
- Finding sex partners through the internet
- Unknown HIV status
- Presence of STIs
- Complacency about the risk of infection
- Forced sex and sexual violence
- Transactional sex
- Substance use before or during sex.

Some of these risk behaviours (finding partners through the internet, use of alcohol and drugs at the time of sex, offering transactional sex, and coercing a partner to have sex) were statistically associated with belonging to the MSM group of respondents.

The survey also highlighted features of the campus environment that might impact on the health of MSM and LGBTI students. These varied from an unaccepting climate, to discrimination based on sexual preference, and threats of abuse or violence. Only half the MSM and non-MSM respondents expressed confidence that institutions would thoroughly investigate instances of discrimination based on gender, race or sexual preference.

While more than half of the MSM group had taken an HIV test on campus, only one-third had used the campus health clinic in the last year, while 40% had used an off-campus health service.

The researchers concluded that institutions should “guard against complacency about the inclusivity of their HIV responses” and design interventions specifically suited to those most at risk, including MSM.

Services appropriate to MSM should be developed and interventions based on the results of the survey – especially those relating to sexual risk behaviour, alcohol and drug use, and institutional leadership on LGBTI issues – should be incorporated into a revised version of the sector’s policy and strategic framework on HIV.

The findings also had implications for the overall character of HIV interventions on campus, the researchers argued. They spoke to the need for a more nuanced approach to HIV and sexual health programmes, which acknowledged the psychological and social meaning of sex to the individual and the contexts in which sexual engagements occur.

The **HIV- and AIDS-related KAB Study of Students and Staff at TVET colleges** was also undertaken in 2014 and



published in 2015. The purpose of the study was to inform programme development for the TVET college sub-sector and to collect baseline data which would assist in the later measurement of programme impact.

The study was undertaken by the Human Sciences Research Council with HEAIDS’ oversight and was funded jointly by NACOSA and USAID. It involved a representative sample of 5 651 students and 1 003 staff members. The sample design permitted the findings to be generalised to the entire TVET sub-sector.

The study revealed that there was a high level of knowledge (upward of 90%) among both students and staff about the sexual transmission of HIV and the effectiveness of condoms in preventing infection. Understanding of mother-to-child transmission, post-exposure prophylaxis and the role of sexual abstinence in prevention was somewhat lower.

However, a high proportion of students and staff expressed negative attitudes to condom use, and these attitudes were particularly strong among men. About half of the male students and staff members said condoms felt unnatural and four out of ten felt condoms changed the sensation of orgasm. About half of staff members indicated that they used condoms every time

they had sex, while nearly 60% of students said they did. There was little difference in the rate of condom use with regular partners and non-regular partners.

A further perspective on condom use was afforded by the information that one in three students had been pregnant or made a partner pregnant – and that 75% of these pregnancies were unintended.

While about one in ten students and staff members preferred anal sex to other forms of sex, there was a lack of awareness of the higher infection risk of anal sex.

Other risk factors were widespread use of alcohol among both students and staff and high rates of age-disparate relationships among students.

Somewhat surprisingly, about 30% of students and 20% of staff were under the impression that HIV could be cured.

Attitudes to antiretroviral treatment were overwhelmingly positive and there was general confidence in the effectiveness of ARVs. While attitudes to people living with HIV (PLHIV) were generally accepting, there was a minority of some 20%–25% of students that responded negatively on most measures of attitudes to PLHIV.

The survey highlighted the under-developed state of HIV programmes and services at TVET colleges. Some access to IEC materials, condoms and HIV testing was only said to be available at a minority of colleges.

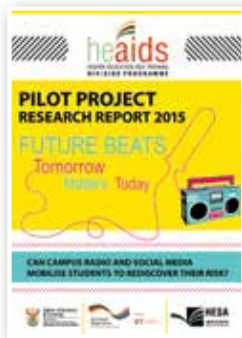
The researchers took the view that the entry of HEAIDS into the TVET sub-sector represented a unique opportunity to invigorate the HIV response, crafting solutions that take account of the diversity of colleges. Key recommendations were to:

- **Strengthen the structural role of the TVET sector in preventing HIV**

The authors pointed out that colleges contribute to HIV prevention simply by providing tertiary education options for large numbers of young people, mostly from low-income families. Research indicates that retaining young people, especially young women, in formal education reduces their risk of HIV infection. It was therefore important for the sub-sector to:

- Sustain and improve its core educational role, achieving high completion and throughput rates, and thereby serving an increasing number of students
- Facilitate better access to the National Student Financial Aid Scheme for students in greatest financial need so that they do not drop out due to financial pressures.
- **Develop wellness clinics for biomedical HIV prevention, care and treatment, paying attention to the specific needs of students and staff.**
Given the life stage of students, it was recommended that there should be strong focus on sexual and reproductive health.
- **Harness the power of health education and communication to stimulate change**
Communication, it was noted, is much more than the provision of information. It is a dynamic, interactive process capable of shaping attitudes, influencing the culture of communities and mobilising people to take action. There is a wealth of local experience in the field of social and behaviour change communication that colleges could tap into.
- **Create a solid, inclusive management team for the development of HIV programmes at college level**
This should involve college management, student leadership, trade union representatives and, possibly, representatives of partner organisations, such as the provincial health department and relevant non-governmental organisations.

The **Future Beats Pilot Project Research Report** aimed to evaluate the impact of the Future Beats



Project on radio station staff and campus HIV units and to establish whether campus radio and social media could contribute to creating awareness of HIV and social drivers of the epidemic.

The research was conducted by the Wits School of Public Health between February 2014

and February 2015 and was published in July 2015 as an ebook which can be downloaded from the HEAIDS

website. It was funded jointly by HEAIDS and GIZ.

The research showed that Future Beats has a number of positive outcomes:

- There were spikes in HIV testing at three university health clinics after the broadcast of radio programmes and the dissemination of social media messaging on HIV testing
- 100% of radio station staff who had participated in Future Beats training sessions showed increased awareness of HIV and greater sensitivity to related social issues
- Listeners to one of the pilot stations, UNIVEN Radio, showed an increase in awareness and knowledge of HIV.

Findings of the Future Beats research were presented at the South African AIDS Conference in June 2015, as well as at the International Conference on AIDS and STIs in Africa (ICASA) later in the year.

The **Situational Analysis or Baseline Study to Understand the Status of HIV Teaching and Learning at TVET Colleges**



was completed in 2015. The purpose of the study was to measure existing delivery, identify instances of good practice and pinpoint barriers to and enabling mechanisms for the expansion of HIV teaching in college programmes.

The study was funded through the NSF grant to the TVET sector and was undertaken jointly by the Centre for Education Policy Development and the SA Institute for Distance Learning.

A sample set of 25 colleges was drawn from all provinces and a combination of quantitative information gathering and in-depth qualitative research was used. A draft research report was circulated internally towards the end of 2015 in order to utilise findings to begin the process of planning curriculum interventions for the sub-sector. However, the final research report will only be launched publicly in 2016.

SECTION 4: COMMUNICATION ACTIVITIES

The Communication Programme at HEAIDS serves to profile both the programme and the issues it addresses in the mass media, to keep stakeholders informed about the programme's work, and to provide marketing and educational collateral for campus activations and other events.

HEAIDS prioritises **outreach to the mass media** as the only truly effective way of reaching the general population nationally, regionally and in specific communities. We believe it is important for the higher education and training sector to be seen to be playing an active role in the national HIV, TB and STI response and that HEAIDS has a role to play in shaping the debates that arise in this area of work.

In 2015, HEAIDS received 204 items of coverage on radio and TV, and in online and print media. This coverage was almost entirely fair and positive in its representation of HEAIDS. Its total value, at advertising rate equivalence, was R5.03 million. For every rand that HEAIDS spent on media liaison in 2015, it received R11 worth of media coverage.

Broadcast media constituted the largest category of media coverage, mainly because radio stations tend to rebroadcast news items several times in a 24-hour cycle and each instance is counted as an item of coverage.

Various activities, such as the national and provincial FTF launches, the release of primary research findings, and participation in major conferences, provided opportunities to engage with journalists. By planning media relations for each event and targeting appropriate media HEAIDS was invariably able to generate interest among journalists. The participation of political leadership and the visual nature of many HEAIDS events added value for the media.

Not only would it be unaffordable for HEAIDS to spend R5 million annually buying advertising space and/or airtime, the independence of journalists means that editorial coverage has greater credibility than paid media coverage.

Stakeholder communication during 2015 occurred through the HEAIDS website, its social media platforms, a new format newsletter (introduced towards the end of the year) and various research and programme publications, as well as through stakeholder participation in launches, events, conference satellite sessions and exhibitions.

HEAIDS undertakes the **writing and production of IEC materials** because access to accurate and relevant information is an essential element in HIV prevention and also drives uptake of available health services. Some institutions have done an outstanding job over the years in producing creative, exciting materials. But not all universities and colleges are equipped to do this and even the best-resourced may not be able to cover the entire range of topics covered by the HEAIDS programmes.

HEAIDS, therefore, produces informative and educational materials as well as materials intended to serve as advocacy tools and marketing items for activations, such as branded caps, T-shirts and memory sticks. A commercial courier is utilised to make two major deliveries of materials to the main campus of every university and TVET colleges each year – at the start of the year and mid-year.

During 2015, the following IEC and marketing materials were produced and distributed:

- Leaflets and posters on FTF, Men's Health Empowerment and Women's Health Empowerment
- Branded pledge walls, base wraps and foam hands for use in FTF activations
- FTF branded T-shirt, memory sticks, caps and pens given away as tokens of appreciation during FTF activations.

Marketing collateral is important in creating visibility for FTF activations, in mobilising student participation in testing and screening, and creating an atmosphere that takes disease prevention out of the clinical setting and into a more reassuring space.

SECTION 5: HEAIDS STRUCTURE

5.1 Members of HEAIDS Advisory Board

Dr M Price (Chairperson), University of Cape Town
Ms H Ntlatleng (Deputy Chair), SACPO
Prof Q Abdool Karim, Centre for AIDS Programmes of Research in SA (CAPRISA)
Dr R Ahluwalia, HEAIDS Director (*Ex officio*)
Ms NL Barnabas, Perinatal HIV Research Unit (PHRU)
Dr V Govender, South African Business Coalition on HIV and AIDS, (SACOPA)
Ms S Letsike, South African National AIDS Council (SANAC)
Mr M Mabizela, DHET
Mr KZ Madzhie, SACPO
Ms V Mafilika, DHET
Prof T Mayekiso, University of Mpumalanga
Mr T Moloja, South African Students Union (SAUS)
Mr TD Moralo, National Education, Health and Allied Workers Union (NEHAWU)
Mr SB Ndlovu, SACPO
Dr P Nevhutalu, Cape Peninsula University of Technology
Mr MC Ramosunya, South African Further Education and Training Student Association
Dr O Seale, Universities South Africa (USAf)

5.2 The National and Provincial Office

Senior Management

Dr Ramneek Ahluwalia, Director
Ms Managa Pillay, Senior Programme Manager: Curriculum Development & Research
Ms Sinikiwe Sithole, Senior Programme Manager: Global Fund

The National Team

Ms Miriam Behrendt, Technical Advisor: Media and Communications
Ms Lorinda Crouse, Executive PA to the Director
Ms Theodora Chauke, Programme Administrator
Ms Bonita du Plessis, Managing Editor
Mr Isaac Makore, Finance and Procurement Officer
Ms Zandile Mashaba, Communication Officer
Ms Mahlatse Modubi, Intern: Curriculum Development
Mr Alex Semba, Programme Manager: Universities
Ms Zamaswazi Shabalala, Programme Administrator
Mr Macdonald Takundwa, Finance and Procurement Officer
Ms Brenda White, Programme Manager: TVET Colleges
Ms Ndinda Zimalirana, Monitoring and Evaluation Officer

The Provincial Team

Ms Christina Dzimiri, Provincial Co-ordinator
Ms Nombuso Madlala, Provincial Co-ordinator
Mr Theophilus Nji, Provincial Co-ordinator
Ms Thandi Ntuli, Provincial Co-ordinator
Ms Thokozani Nyawasha, Provincial Co-ordinator
Ms Marilize Olckers, Provincial Co-ordinator
Ms Gail Roman, Provincial Co-ordinator
Mr Pakiso Thulo, Provincial Co-ordinator

SECTION 6: FINANCIAL REPORT FOR 2015

6.1 Income and expenditure

HEAIDS income for the 2015 year amounted to R35.2 million and R27.6 million of this amount, or 79% of the total, was spent during the course of the year, resulting in a surplus of R7.6 million.

The sources of income and main categories of spending are reflected in the tables below.

Income 2015

Source of income	Amount
Global Fund grant	R12 328 706
DHET allocation	R7 700 000
NSF grant for Curriculum Development	R13 332 500
Membership fees from institutions	R1 100 000
GIZ grant for Future Beats	R766 300
Total income	R35 227 506

Expenditure 2015

Cost centre	Actual spending	Budget allocation
Programme implementation	R12 348 385	R18 734 802
Co-ordination of activities	R3 352 344	R4 023 169
Monitoring and evaluation	R3 148 337	R3 428 337
Human resources and administration	R8 819 150	R9 041 198
Total expenditure	R27 668 216	R35 227 506

Total surplus	R7 559 290
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6.2 Observations on sources of income

In 2015, nearly two-thirds of income (63%) was public sector funding comprising the allocation from the National Treasury to the DHET, the NSF grant for curriculum development, and membership fees paid by public universities and colleges. This represented a significant shift away from dependence on donor funding for HEAIDS and progress in terms of diversifying funding.

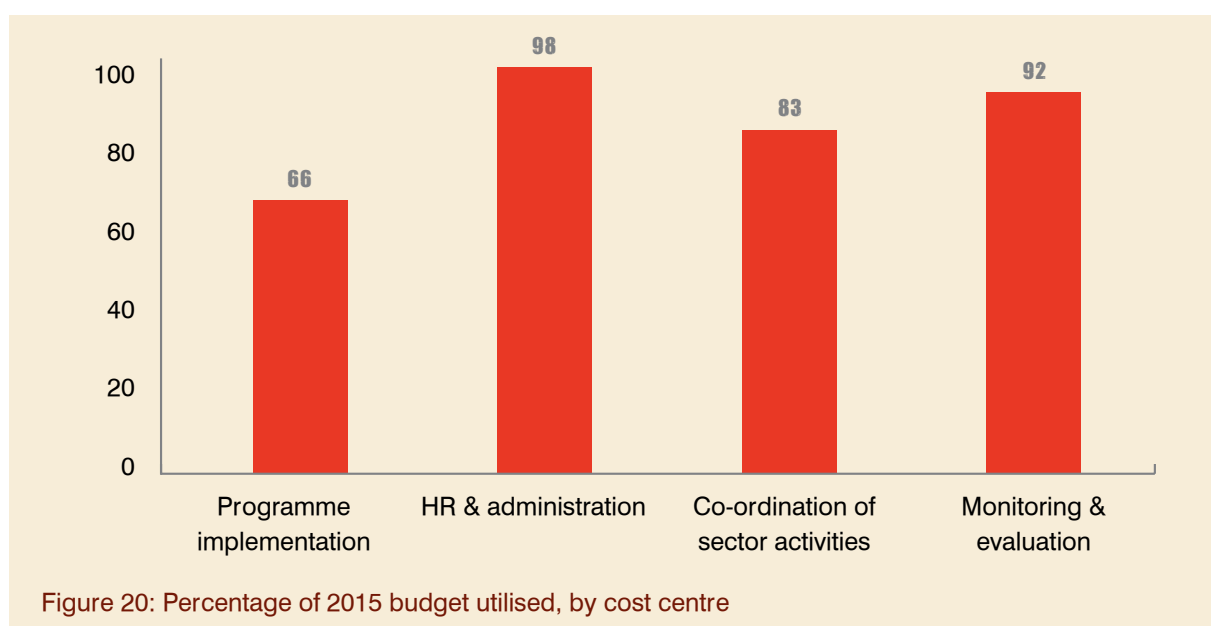
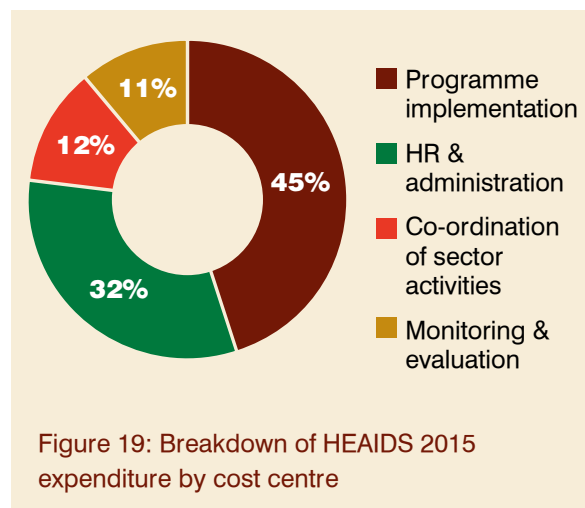
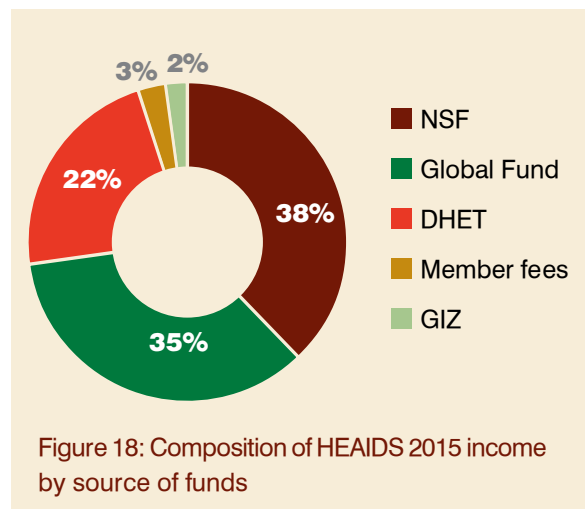
6.3 Patterns of expenditure

Figure 18 indicates the contribution of various categories of expenditure to total programme spending. Appropriately, programme implementation constituted the single largest spending category at 45% of the total. Spending on human resources and general administration accounted for nearly one-third of expenses.

Overall, nearly 80% of the budgeted amount for 2015 was spent during the course of the year. Underspending against the budget varied considerably by cost centre, as indicated in Figure 19, and was most pronounced in relation to programme implementation where only 66% of the allocated budget was spent. This was largely due to the fact that spending against the NSF grant for curriculum development was slow initially. However, once the preparatory work was done and academics were delivering and participating in training sessions and workshops, funds began to be utilised at a much greater rate.

6.4 Auditing of the HEAIDS Programme

Fiduciary responsibility for the HEAIDS Programme vests in Universities South Africa (USAf). Accordingly, the programme is audited as part of the USAf audit and any opinion expressed by the auditors on HEAIDS' financial management will be included in the broader audit report.







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